PATIENT LINKAGE, RETENTION AND RECOVERY PROTOCOL



MINISTRY OF HEALTH & WELLNESS

National HIV Programme

Patient Linkage, Retention and Recovery Protocol

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LIST OF ACRONYMS AND ABBREVIATIONS

AC	Adherence Counselor
ART	Antiretroviral Therapy
ARV	Antiretroviral
BCC	Behaviour Change and Communication
CI	Contact Investigator
DOT	Directly Observed Treatment
ETC	Entry to Care
HATS	HIV/AIDS Tracking System
HPO	Health Promotion Officer
HSTU	HIV/STI/TB Unit
LTFU	Loss to Follow-Up
MO(H)	Medical Officers of Health
MOHW	Ministry of Health and Wellness
MSW	Medical Social Worker
MSW-CM	Medical Social Worker - Case Manager
NGO	Non-Governmental Organisation
NSU	National Surveillance Unit
PITC	Provider Initiated Testing and Counselling
PLHIV	Persons Living with HIV
PTC	Post Test Counsellor
QA	Quality Assurance
RGD	Registrar's General Department
RHA	Regional Health Authority
RME	Regional Medical Epidemiologist
TCS	Treatment Care and Support
TCSO	Treatment Care and Support Officer
TRAT	Treatment Readiness Assessment Tool
TSIS	Treatment Services Information System

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OVERVIEW

Jamaica at the end of 2023 had an estimated HIV prevalence of 1.3% which represents approximately 30000 persons living with HIV (PLHIV). At the end of 2023, the national HIV treatment continuum of care reflected 91% of persons estimated diagnosed, 71% of persons linked to care that have been diagnosed, 54% of persons diagnosed on ARVs, and 43% of persons diagnosed being virally suppressed. Presently the largest gap in the treatment cascade is linkage to care and the second largest gap is retention on ARVs which poses a threat to treatment efforts. PLHIV not engaged in care and on treatment are not able to achieve viral suppression and run the risk of transmission of HIV to other persons. Based on the continuum of care, there is a significant fall off in linkage to care of those diagnosed, and retention in care of those persons engaged indicating that there are a number of PLHIV not accessing HIV treatment care and support services. The formalization and merging of documents to reflect the management of these processes is needed to ensure that the continuum of care is improved and the PLHIV attains viral suppression thus contributing to HIV epidemic control.

The aim of the manual is to formalize the standard operating procedures in treatment sites with regard to the management of PLHIV patients being linked, retained on antiretrovirals and returned to care from being out of care (recovery processes). The guidelines are designed to link, monitor and re-engage patients who miss clinic appointments, as well as patients who are deemed to be 'lost to follow up' (LTFU), defined as patients who have not accessed treatment and care at a treatment site within 90 days of missing a scheduled appointment.

The initial retention and recovery guidelines were launched in 2013 and have since been updated based on recommendations from additional patient re-engagement campaigns in 2018. The Entry to Care (ETC) protocol was developed in April 2020 with a view to determine the disposition of those persons never linked to care and ultimately complete the linkage to care process. In addition to the referral and linkage guidelines these protocols have been combined to complete the process from linkage to care to viral suppression and the engagement of persons out of care to return to care. The agreed upon previous guidelines have proven to have a positive impact in linking, retaining and returning more patients to care within a shorter period and have contributed to the reduction of missed appointment rates at the sites.

The protocol will commence with the Referral and Linkage process and continues on to the ETC process and retention protocol to prevent missed appointments and LTFU and concludes with the recovery process of persons out of care.

Guiding principles

- **CONFIDENTIALITY AND INFORMED CONSENT:** The patient agrees to HIV testing by informed consent. Information shared during the counselling or referral and linkage must not be disclosed to individuals other than the health care providers directly involved in providing services to the patient. The HIV test result must only be reported to the patient unless the patient states the desire to share the test result with a family member, partneror close friend.

- **PATIENT CENTRED:** A patient-centred approach should be used while conducting counselling, patient assessment, referral and linkage and follow-up. A patient's preferences and constraints should be duly considered when exploring referral and linkage options. The most convenient options for the patient should be taken provided that option is feasible and does not negate HIV care and treatment guidelines.

- **CONTINUUM OF CARE:** Continuum of care is a concept involving an integrated system of care that guides and tracks patient over time through a comprehensive array of health services. For HIV care and treatment, the continuum starts with HIV diagnosis, followed by referral and linkage to care, retention, initiation of antiretroviral therapy, and achievement of viral suppression. The fundamental principle is to ensure that each service component within the continuum of HIV care and treatment is integrated and patients are retained throughout.

ENTRY TO CARE PROTOCOL

It is intended that the Entry to Care document will provide the standard operating procedures in linking individuals who have been confirmed positive for HIV into care. The protocol should be utilized for persons diagnosed with HIV during specified time periods.

Aim:

To link all persons diagnosed with HIV to appropriate treatment services

Objectives:

- Identify persons diagnosed HIV positive from screening services (Outreach/ PITC/ STI Clinic) to HIV care.
- Link persons identified from screening services, surveillance and contact tracing to care.
- Document the final disposition of persons diagnosed with HIV.

The linkage to care process is paramount to retaining people on antiretrovirals, achieving and maintaining viral suppression and preventing PLHIV from being lost to follow up. Integral in the ETC protocol is the partnerships with key stakeholders who help to achieve the objectives of linkage to care. These include the regional health authorities (RHAs), parish health departments, civil-society organisations that offer treatment care and support services, and community led-organisations who offer critical services in engaging and empowering members of the community.

Roles and Responsibilities

The following tables list the staff roles and responsibilities that are integral to the success of the ETC process.

Table 1: Roles and Responsibilities of Parish Staff for Entry to Care Protocol

Staff	Role	Responsibility
Medical Officers of Health	Manages the Primary Health Care Programmes and Public Health at the Parish level.	• Monthly review of HIV indicators for linkage and ETC • Liaise with Reg. HIV Coordinator to design and implement initiatives to increase linkage to care at the service delivery level according to the National Protocol
Community Peer Educators	Conduct outreach activities at the parish level	• Link positives identified with the Contact Investigator to ensure linkage to treatment
Behaviour Change Communication Officers	Coordinate and implement the prevention programme at the parish level	• Monthly review of programmatic activities and implementation for linkage • Monthly review of HIV indicators for prevention as it pertains to linking newly diagnosed PLHIV to care • Liaise with Reg. BCC Officer (HPO) to design and implement linkage initiatives
Community Facilitator	Facilitates linkage and retention of peers	• Provides support to newly diagnosed PLHIV up to viral suppression • Provides monthly updates on peers linked to care
Contact Investigator	Coordinates the case investigation, contact tracing and subsequent testing and	• Conduct pre and post-test counselling (if he/she is the tester)

Staff	Role	Responsibility
	referral of sexually transmitted infections and all notified cases of HIV.	• Review all notifications from the Parish Surveillance team and ensure all persons are contact traced and linked to care • Generate a Confidential Reporting Form and submit to the Medical Officer of Health for the parish and the Ministry of Health and Wellness Surveillance Unit • Conducts laboratory surveillance to ensure notifications are done and contact tracing and linkage occurs • Ensures that STI clinic attendees are screened for HIV & Syphilis • Conducts ongoing surveillance of existing PLHIV in care to ensure that partners are screened according to the national protocol • Coordinates with the prevention team to ensure notifications are done and contact tracing and linkage occurs • Implement initiatives to increase linkage to care at the service delivery level according to the National Protocol • Document all linkage and ETC data along with the TCS team • Prepare monthly reports with relevant HIV data on linkage

Staff	Role	Responsibility
Treatment Care & Support Officer	Coordinate site level activities for treatment care & support	• Document all linkage and ETC data along with the TCS team • Prepare monthly reports with relevant HIV data on linkage • Contribute to monthly/ quarterly HIV/STI/TB reports • Implement initiatives to increase linkage to care at the service delivery level according to National Protocol • Develop site specific plans along with TCS team to improve HIV treatment outcomes

Table 2: Roles and Responsibilities of Regional Staff for Entry to Care Protocol

Staff	Roles	Responsibility
Regional Technical Director	Manages the HIV programme in the region	• Quarterly review of HIV indicators for linkage to HIV care.
HIV/STI/TB Coordinator	Coordinates HIV programme between the Parishes in the designated RHA	• Monthly review of HIV indicators - linkage to HIV care • Coordinate with other HIV Coordinators. in other RHAs migratory cases between regions for linkage. • Design initiatives to increase linkage to care at the service delivery level according to National Protocol • Report quarterly on linkage and ETC through the quarterly HIV/STI/TB report

Staff	Roles	Responsibility
Regional Contact Investigator	Coordinates the CI programme in the region	• Creates and collates master list for the RHA in collaboration with Senior Parish CI • Conducts monthly meetings with CIs to ensure the fidelity of the data within the region • Liaises with RME and NSU on probable death from the NSU. • Develops initiatives alongside the HIV Coordinator to improve linkage in the RHA.
Regional Treatment Care & Support Officer	Coordinate regional activities for treatment care & support	• Review linkage and ETC data along with the Parish TCSOs • Prepares monthly reports with relevant HIV linkage and ETC data • Implement initiatives to increase linkage to care at the service delivery level according to National Protocol in conjunction with the regional CI. • Develop regional plans along with Regional team to improve linkage to HIV care

Staff	Roles	Responsibility
Regional Behaviour Change Coordinators	Coordinate regional activities for the prevention programme	• Monthly review of programmatic activities and implementation for linkage • Monthly review of HIV indicators for prevention as it pertains to linking newly diagnosed PLHIV to care • Develop regional plans along with Regional team to improve linkage to HIV care

Table 3: Role and Responsibilities of National Staff for Entry to Care Protocol

Staff	Roles	Responsibility
Monitoring and Evaluation Officer	Provide technical assistance in monitoring and evaluation and support the regular analysis of datasets. Conducts data analysis and data quality verifications for validity, reliability and accuracy of databases used.	• Quarterly reports on indicators to monitor progress of the ETC process. • Data verification of the national HIV treatment database for valid and reliable information on persons diagnosed with HIV. • Provide technical assistance in data collection and analysis.
HIV/STI Information Officer	Establish and maintain the national HIV treatment database.	• Manage the national HIV treatment database and aid field staff in the use of the database for daily activities with regards to ensuring all data is present and correct for linkage.

Staff	Roles	Responsibility
Director Treatment Care & Support	Manages the development, implementation, monitoring and evaluation of national policies and protocols for HIV, STI & TB	• Monthly review of national picture as it pertains to linkage. • Provide feedback based on reports, QA activities and implementation activities for linkage • Updates protocols and policies in keeping with international best practices and country context to improve linkage to HIV care
Programme Development Officer HIV	Coordinates the development, implementation, monitoring and evaluation of national policies and protocols for HIV	• Reviews and provides feedback on monthly and quarterly regional reports on linkage • Conducts quality assurance activities for HIV for linkage • Reviews and provides feedback on initiatives to increase linkage to care at the service delivery level according to National Protocol • Develops training programmes in keeping with international standards for HIV for entry to care

Staff	Roles	Responsibility
Programme Development Officer STI	Coordinates the development, implementation, monitoring, and evaluation of national policies and protocols for STI	• Conducts quality assurance activities for STIs - ensuring linkage to appropriate services • Reviews and provides feedback on initiatives on STI management, contact tracing and linkage at the service delivery level according to National Protocol • Develops training programmes in keeping with international standards for STIs and linkage to HIV treatment & preventative risk reduction activities.
Programme Officer HIV (National TCSO)	Monitors HIV treatment implementation	• Reviews and provides feedback on monthly and quarterly TCSO reports for linkage • Provides feedback on initiatives to increase linkage to care at the service delivery level according to the National Protocol • Review site specific plans along with TCS team to improve HIV treatment outcomes through the linkage process at treatment sites.

Entry to Care Protocol Process

The Entry to Care Protocol process is divided into three phases:

- Phase 1(Identification Phase): The identification of patients
- Phase 2 (Preparation Phase): Preparation of patients to be entered into care
- Phase 3(Entry to Care Phase): The actual entry of patients into care.

Identification Phase

This phase entails gathering the relevant information of the individuals to be entered into care. The most accurate and reliable information is used to identify those individuals and these include surveillance data, laboratory data, contact investigator field notes and confidential reporting forms. Furthermore, the sharing of patient data across parish surveillance systems, treatment sites and support services will enhance the quality of data collected. Valid and reliable information is critical and will form the basis for phase 2. The procedure is outlined in the flowchart and table below.

Table 4: Activities for the Identification Phase of the Entry to Care Protocol

Procedure	Responsible Officer	Description
Collect testing information from health facility labs	Contact Investigators	Examine laboratory log books for HIV diagnoses for the prescribed time period and complete the lab data collection tool (Appendix 1)
Cross match information from the laboratory logs with CI notes/records	Contact Investigators	Utilize CI notes/records to add missing or relevant information to the laboratory data collection tool.
Cross match information with Treatment Database	Contact Investigators, Treatment, Care and Support Officers	Utilize TSIS to determine linkage status of persons on the laboratory data collection tool.

Procedure	Responsible Officer	Description
Cross match information with NSU Probable Deaths List/Death Registries	Contact Investigators	Utilize the NSU probable death lists and death registries to determine the disposition of persons on the laboratory data collection tool.
Request cross checking with the complete national treatment database	Treatment, Care and Support Officers	Generate a list of names to be checked in the complete national treatment database.
		Liaise with Data Managers from private healthcare facilities within the parish to determine if PLHIV are accessing care privately.
	Regional HIV/STI/TB Coordinator	Submit list of names to be cross checked to the Strategic Information Component at the National Level
	Monitoring & Evaluation Officer, HIV/STI Information Officer	Complete cross checking and provide information to the regional team.
Generate Line Listing of persons to be contacted	Contact Investigators	Generate line listing of all persons who were identified from the lab books but were found not linked to care and not located in the NSU presumed death list or death registries (Appendix 2). This will form the basis of activities in phase 2. Utilize CI notes/records to add missing or relevant information.

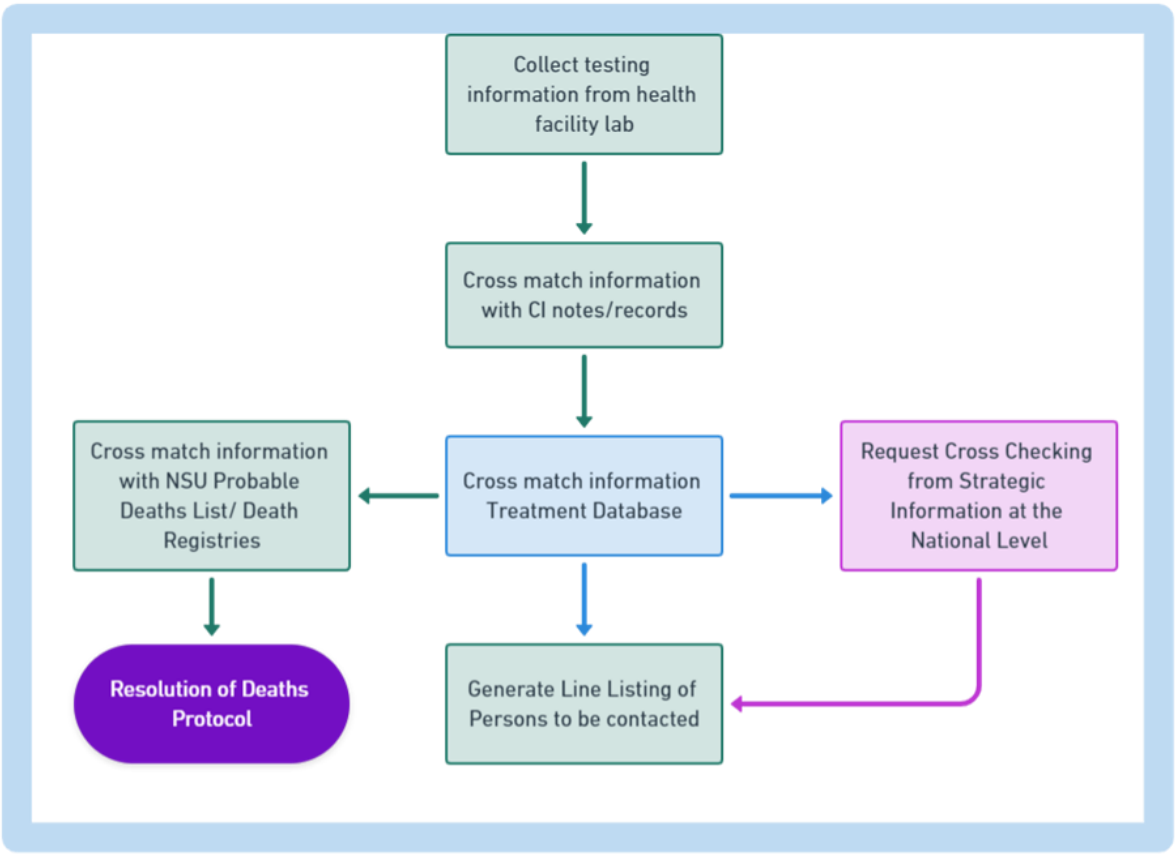


Figure 1: Process Flow Chart for Positives Identified in the Entry to Care Protocol

Preparation Phase

This phase entails preparing individuals to enter into care. The contact investigators will coordinate the activities in the preparation phases. Other members of the Treatment Care and Support team that are critical to this phase are Medical Social Workers, Case Managers, Community Facilitators and Community Health Aids. Contact Investigators using the lists generated in phase 1, will attempt to locate the individuals and complete the linkage protocol. The contact log sheet (Appendix 3) will be used to document all attempts to locate PLHIV. A person is considered unable to locate after unsuccessful attempts to locate (8 phone calls and 3 home/field visits). Each case should be worked on for a total of four (4) weeks. During this phase the contact investigators will record the disposition of all PLHIV on the generated line listing.

Dispositions may be categorized as follows:

- Located
    - Alive and linked
    - Alive and not linked
    - Migrated
    - Confirmed Death
- Not Located
    - Unable to Locate
    - Unlocatable due to poor identifying information
    - Presumed Death
- Other (Please State)

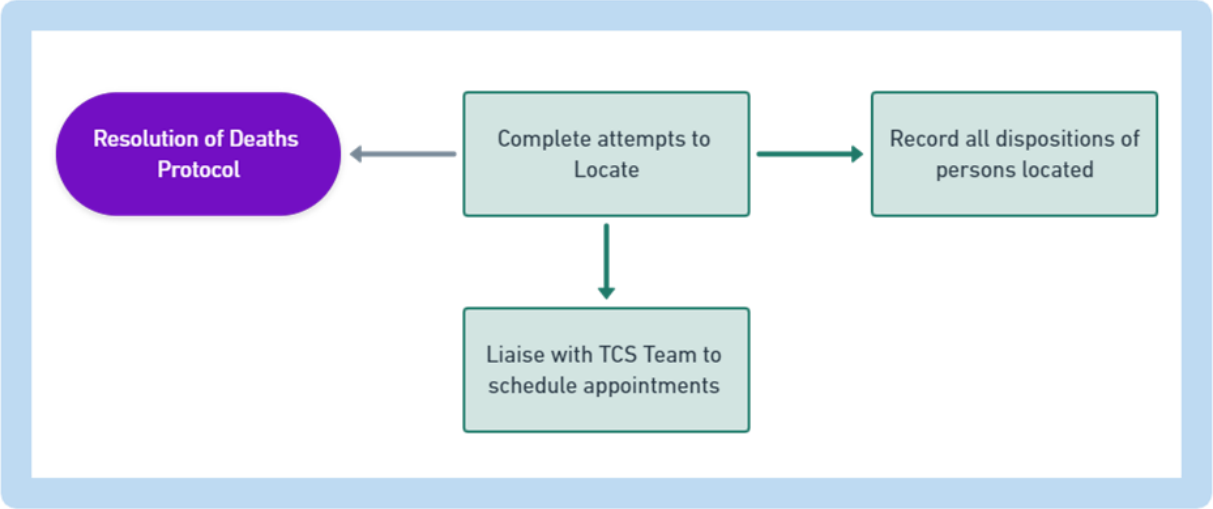


Figure 2: Process Flow for Disposition of Locating Attempts for the Entry to Care Protocol

Entry to Care Phase

At the end of this phase, the individual has successfully been linked to care. It is critical that the patient is closely monitored by the medical and psychosocial team. The inclusion of peer support from diagnosis is necessary to ensure retention in care and sustained viral suppression.

Patients should not be limited to established HIV treatment sites in this phase. Through the integration of HIV in Primary Health Care, all health facilities offering HIV care should be considered, especially if stigma and discrimination were cited as barriers to entry into care. The activities in this phase are expected to take 8 weeks.

Table 5: Activities for Entry to Care Phase of Entry to Care Protocol

Appointment Schedule	Activity	Cadre of Staff	Comments
First appointment	Contact Investigator Assessment	Contact Investigator	This is usually the first contact the patient has with the treatment team and the CI should elicit the patients correct contact information, address and next of kin, as well as sexual contacts.
	Social assessment	Medical Social Worker, Medical Social Worker – Case Manager	This is done to address barriers to linkage and retention
	Psychological assessment	Psychologist	
	Peer Support	Community Facilitator	This is critical to ensure that the patient is provided with ongoing support and in turn is mentored to provide the same support to other newly diagnosed PLHIV
	Clinical examination (Please reference the Clinical Management of HIV Disease Manual)	Medical Practitioner	This may be abbreviated for the first appointment in cases where the patient needs to be "fast tracked" to ensure retention in care. All patients should be encouraged to start ART at diagnosis but if the patient is unwilling this may be deferred to the next appointment. This visit must include referring the patient for a Mantoux test, Urinalysis, Mammogram in women over 40, Conducting or requesting a Pap Smear or Anal Smear where indicated
First Appointment	Sample collection for necessary labs as per the National Guidelines (Please reference the Clinical Management of HIV Disease Manual)	Medical Technologist, Lab Technical Assistant, Phlebotomist	These samples should be expedited for processing
	Adherence assessment	Adherence Counsellor	

Appointment Schedule	Activity	Cadre of Staff	Comments
	Pharmacy pick up	Pharmacist	The patient should be counselled on the importance of adherence to ARV drugs and their side effects
2 weeks from first appointment (consider telehealth visit)	Clinical examination	Medical Practitioner	More detailed history and clinical examination if not done at the first visit. If a comprehensive history & examination was completed, this patient may benefit from a telehealth visit at two weeks.
	Adherence assessment	Adherence Counsellor	Adherence counselling continued
	Social assessment	Medical Social Worker, Medical Social Worker – Case Manager	Continued assessment of barriers to retention
4 weeks from second appointment	Clinical examination	Medical Practitioner	Assessment with test results
	Adherence assessment	Adherence Counsellor	Adherence counselling continued
	Social assessment	Medical Social Worker, Medical Social Worker – Case Manager	Continued assessment of barriers to retention
6 weeks from third appointment (consider telehealth visit)	Clinical examination	Medical Practitioner	Assessment with test results
	Adherence assessment	Adherence Counsellor	Adherence counselling continued
	Social assessment	Medical Social Worker, Medical Social Worker-Case Manager	Continued assessment of barriers to retention
Monthly (telehealth vs. in person can be decided on) until the patient is suppressed	Clinical examination	Medical Practitioner	Clinical assessment
	Adherence assessment	Adherence Counsellor	Adherence counselling continued
	Social assessment	Medical Social Worker, Medical Social Worker – Case Manager	Continued assessment of barriers to retention
	Sample collection for necessary labs as per the National Guidelines	Medical Technologist, Lab Technical Assistant, Phlebotomist	These samples should be expedited for processing

Appointment Schedule	Activity	Cadre of Staff	Comments
	Pharmacy pick up	Pharmacist	The patient should be counselled on the importance of adherence to ARV drugs and their side effects

Resolution of Unlocated PLHIV Protocol

The National Surveillance Unit monitors the HIV/AIDS Tracking System with approximately 27,000 persons diagnosed with HIV. The TSIS database has approximately 20,000 persons linked to care. The deficit of 7000 persons not linked to care continues to be an ongoing data challenge that requires resolution at the field level. After all efforts have been exhausted for ETC the protocol is to be executed.

The following steps are to be carried out to resolve deaths at the parish level:

1. Death registry at the hospitals within parishes must be reviewed.
2. For persons not located with family members indicating death - this must be confirmed by the death registry and/or RGD through NSU.
3. Confidential reporting forms are to be updated to reflect change in status and submit to the NSU

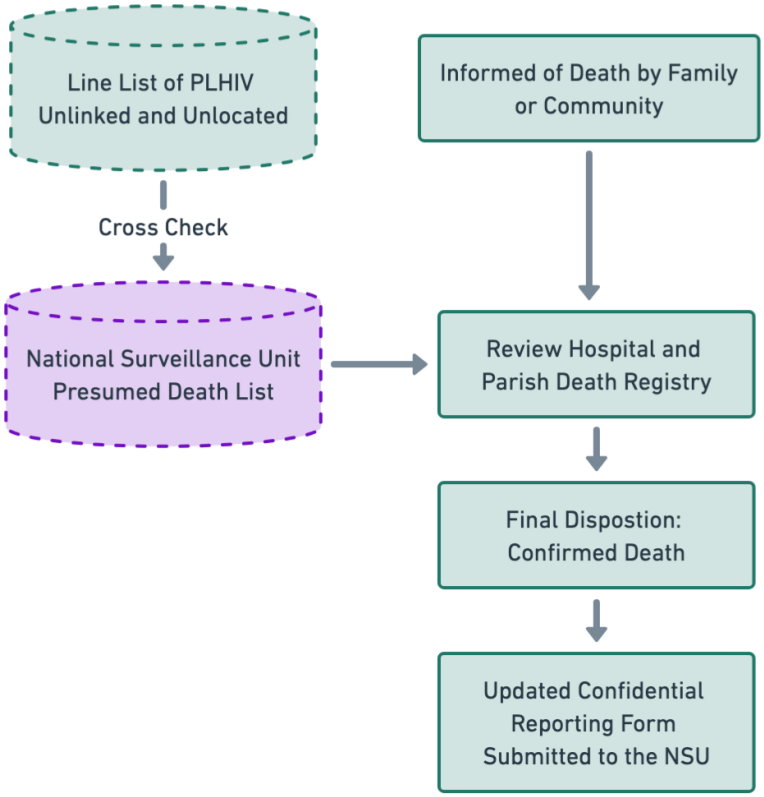


Figure 3: Resolution of Death Protocol Flow Chart

Monitoring and Evaluation of the Entry to Care Process

Strategic information is required to measure progress and provide evidence for decision-making. The M&E for the Entry to Care Protocol, outlined below, should be incorporated in routine national HIV programme monitoring and evaluation.

Table 6: Monitoring and Evaluation Indicators for the Entry to Care Protocol

Indicator	Overview	Numerator	Denominator	Data Collection Methodology
Percentage of persons diagnosed with HIV not linked to treatment and care services.	This indicator is key to measuring the linkage protocol. People who are not linked to treatment and care services are those who have not had a first clinic visit at any public or private treatment facility.	The number of people who are not linked to any treatment and care services	The number of persons diagnosed with HIV	**The numerator** is generated after a cross checking the laboratory data collection tool with the national treatment database (to include both public and private facilities). Those persons not found in the database are considered unlinked. **The denominator** is counted from the line listing generated from the laboratory investigations.
Percentage of PLHIV unlinked who are locatable	This indicator aims to measure the efficiency of data collection at the point of diagnosis in order to facilitate the location of PLHIV and link them to care. A number of persons with	The number of PLHIV unlinked with sufficient locatable information.	The number of people who are not linked to any treatment and care services	**The numerator** is generated by counting all PLHIV with complete information after cross checking with the CI field notes and recorded in the line listing including First

Indicator	Overview	Numerator	Denominator	Data Collection Methodology
	a positive HIV test have insufficient identifying information.			Name, Last Name, Phone Number, Address, Date of Birth, Sex at Birth. **The denominator** is generated after cross checking the laboratory data collection tool with the national treatment database (to include both public and private facilities). Those persons not found in the database are considered unlinked.
Percentage of PLHIV located	This indicator will measure the progress in the investigation process to identify and locate PLHIV who need to be linked to care.	The number of unlinked PLHIV located.	The number of PLHIV unlinked with sufficient locatable information.	**The numerator** is generated by counting the number of persons located during the course of the case investigations. All attempts to locate PLHIV who need to be linked to care and the successful location of these persons should be recorded in the contact log sheet. **The denominator** is generated by counting

Indicator	Overview	Numerator	Denominator	Data Collection Methodology
				all PLHIV with complete information after cross checking with the CI field notes and recorded in the line listing including First Name, Last Name, Phone Number, Address, Date of Birth, Sex at Birth.
Percentage of PLHIV linked of those who were not linked to treatment and care services	This indicator measures the entry to care process in locating and successfully linking PLHIV who were previously not linked to care.	The number of PLHIV successfully linked to care.	The number of PLHIV unlinked who were located and alive.	**The numerator** is generated by counting the number of persons located and alive after case investigations with a first clinic date on the national treatment database (TSIS). **The denominator** is generated by counting the number of PLHIV that have been located and found to be alive during the course of the ETC process. A person is said to be located and alive if there was direct communication (phone call, field/home visit)

Indicator	Overview	Numerator	Denominator	Data Collection Methodology
				with the person of interest.
Number of unlinked PLHIV with completed investigations	One of the objectives of the ETC process is to have a final disposition of all PLHIV in Jamaica. This indicator provides the number of cases investigated and receiving a final disposition.	The number of cases that received a final disposition.	N/A	This indicator is generated by counting all PLHIV previously unlinked with a final disposition. The dispositions to be counted are: • Alive and Linked • Alive and Not Linked • Migrated • Confirmed Death
Number of unlinked PLHIV with unresolved cases	This indicator provides the number of cases with unresolved investigations.	The number of cases without a final disposition	N/A	This indicator is generated by counting all PLHIV previously unlinked with unresolved cases. The dispositions to be counted are: • Unlocatable due to poor identifying information • Unlocated • Presumed Death

Data Sources

1. Laboratory Logbooks
2. Contact Investigator Field records/notes.
3. HIV/AIDS Tracking System (HATS)
4. Treatment Sites Information System (TSIS 2.0)
5. Line listing of PLHIV not linked to care.
6. Contact Log Sheet

Digital Data Collection

The use of digital data collection can provide convenience in reporting and reduce data entry/transcription errors. CI tablets should be equipped to collect all the necessary information from their case investigations. (Appendix 2). These tablets are to be password protected and stored in secure environments. The data collected will be housed on the servers within the Ministry of Health and Wellness. This system allows for the production of data tables to be used in quarterly reporting.

Frequency and Responsibility of Reporting

Data should be collected continuously at the parish level and aggregated quarterly. Data collection is to be carried out by the contact investigators and data reviews are the responsibility of the Parish Medical Officer of Health (MO(H)). The Regional HIV/STI/TB Coordinator will report on the indicators quarterly to the national level.

REFERRAL AND LINKAGE PROCESS

The referral and linkage processes start with an HIV positive test result in a treatment facility, a non-treatment facility, or an outreach setting. The processes are described in this section and are categorized into four phases: **Identification**, **Preparation**, **Outcome**, and **Follow- up**.



Post-test counselling for HIV positive persons:

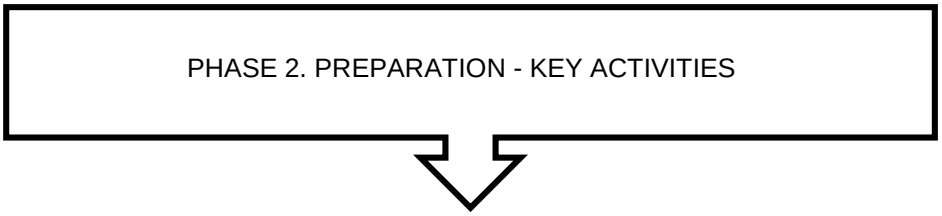
Counselling for persons with HIV-positive test results aims to provide:

- Psychosocial support to cope with the emotional impact of the test result
- Facilitate access to treatment, care and prevention services
- Disclosure to sexual partners and the prevention of transmission.

This should happen immediately after testing.

Notification

The process of informing the Parish, Regional and National Disease Surveillance System when a new case of HIV is diagnosed. This should occur within 24 hours as HIV is a Class 1 Disease. **It is critical to note that the Class 1 Notification Form should be generated even if the individual is asymptomatic**.



Identifying appropriate facility

Identify the HIV care and treatment facility that the newly diagnosed HIV positive person can be linked to. If the HIV testing facility is a HIV care and treatment facility, it is advisable to link the patient to the same facility. If not, referral should be made to appropriate facilities considering location, patient's preference, and other factors as required. The identification of an appropriate treatment site should be done on the same

day of testing.

Referral

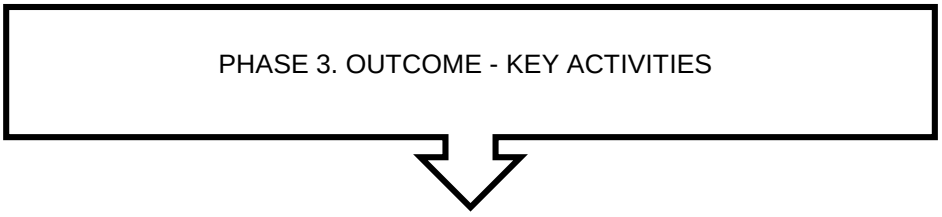
As described above, if the HIV testing facility does not provide HIV care and treatment services, or if testing was conducted in an outreach setting, a referral is made to an appropriate HIV testing site for a confirmatory HIV test as soon as possible. A referral form containing details of the patient is generated and given to the patient from the referring facility to submit to the referral facility by the patient. The tester should also make contact with the point person at the referral facility. The referral process should include helping patients connect to services. This includes setting up of appointments, giving directions to the facility, sending contact details of the patient to the referral facility and following-up to ensure contact between the patient and the service providers in the referral facility.

Patient assessment

Orient the patient on HIV care and treatment services and conduct a review of the patient's situation. This will be done to identify barriers that the patient may face in getting follow-up care such as: *location of treatment site (referral), financial constraints, substance abuse or mental health issues*. An assessment will be conducted to develop a plan in collaboration with the patient to address barriers identified using the resources available to the HIV treatment facility and the patient's personal resources. These activities should occur on the same day of HIV testing.

Same day appointment scheduling

Scheduling an appointment date for a doctor's visit and CD4 test for a newly diagnosed HIV patient on the same day of HIV testing. If appointments are not scheduled on the same dayof HIV testing particularly for those patients who need to be referred to a different facility, appointment scheduling should be done no later than on the first day of their referral visit.



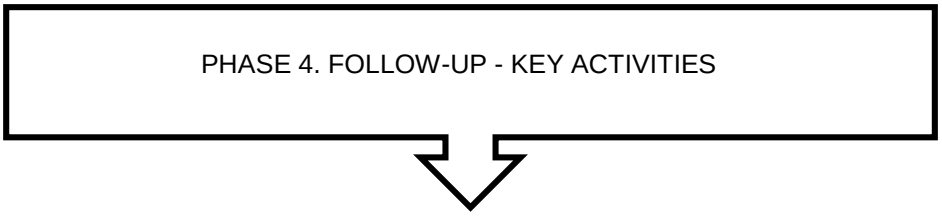
Linkage

Linkage to medical care is the desired outcome of the referral processes. This should result in a person being

seen by a health-care provider (e.g. Physician, nurse practitioner) to receive medical care for his/her HIV infection. For this protocol, a person is said to be linked if he/she attends his/her first doctor's appointment or if he/she meets the admissions and linkage criteria. In the event that a person receives an HIV positive diagnosis during a period of admission at a non-HIV treatment site facility, he/she may be considered linked to care once the following Admissions and Linkage Criteria have been met:

1. Diagnosed and started on ARVs during admissions
2. Seen by members of the HIV Treatment team (Medical Doctor and Social Worker) from the closest assigned treatment site and the Intake Tool completed
3. Entered on TSIS database (Social Worker)

Linkage should be completed within 2 weeks of being diagnosed with HIV. Close follow-up should be done at minimum for the first six months after diagnosis to ensure that the patient is retained in care.



Monitoring linkage

Continuous monitoring of the referral and linkage processes using the key performance indicators. (refer to section on measuring linkage).

Referral feedback

A process of sending feedback from one facility to another regarding the post referral status/outcomes of a patient. The referral facility should complete the referral feedback form (same form used in health facilities) and send it to the referring facility within a week after referral.

Referral tracking

A process of requesting information from a referral facility by a referring facility on the outcome of a referral. This also includes follow-up with the patient by the referring facility to ensure that the patient has been linked and is getting the attention at the referral facility.

Standards of Service Delivery

Standards of service delivery outline the minimum set of activities to be performed, the staff responsible and the measurement of the completed activities.

Table 7: Standards of service delivery for referral, linkage and retention processes

LINKAGE LEVEL	TYPE OF SERVICE	RESPONSIBLE STAFF	MEASURE OF COMPLETION
IDENTIFICATION	- Post-Test Counselling - Generate Class 1 Notification	Tester, CI, other PTC (Government, NGOs, Private)	Class 1 Notification reaches MOH
PREPARATION	- Identify appropriate facility	CI, other PTC (Government, NGOs, Private)	- Patient linked with appropriate facility
	- Referral if required	CI, other PTC (Govt., NGOs, Private)	- Patient reaches referral facility
	- Same day appointment scheduling: for first doctor visit and CD4	Contact Investigator	- Patient gets appointment on the same day of testing
	- Assessment for barriers to linkage (medical, social and emotional) of patient	Medical Social Worker, Medical Social Worker- Case Manager, Psychologist Adherence Counsellor	- Patient linked to social support services
OUTCOME	- Linkage to treatment site (first doctor's visit and CD4)	Combined effort of all CI, MSW, MSW-CM, AC, and others	- Patient attends first doctor's visit at treatment site - Patient does CD4
FOLLOW-UP	- Monitoring	Contact Investigator	- Linkage measured using performance indicators
	- Follow up with patient to ensure that appointment is kept - Track Lost to Follow up	Adherence Counsellor	Patient does not miss appointments
	- Assist patient to adhere to treatment regime	Adherence Counsellor	Patient adheres to treatment regime
	Referral feedback and referral tracking	Contact Investigator,	Outcome of referral is documented

RETENTION & RECOVERY PROTOCOL

The process of retention starts with diagnosis. The PLHIV has to be linked to care and develop a culture of adherence to clinic appointments, laboratory testing, psychosocial support (interventions) and medication. pick-ups. Whilst linkage to care is the first step, retention in care is necessary to achieve and maintain viral suppression. There are several persons that contribute to retention, however the psychosocial team are the primary group responsible for ensuring retention.

Aim:

To retain all persons diagnosed with HIV in appropriate treatment services

Objectives:

- Reduce the rate of missed appointments among PLHIV in care.
- Engage PLHIV who are out of care and return them to care.
- Standardize protocols for retention and recovery activities that produce a consistent and efficient approach in reducing lost-to-follow-up.

Roles and Responsibilities

The following tables list the staff roles and responsibilities that are integral to the success of the retention and recovery protocol.

Table 8: Roles and Responsibilities of Parish Staff for the Retention and Recovery Protocol

Staff	Role	Responsibility
Medical Officers of Health	Manages the Primary Health Care Programmes and Public Health at the Parish level.	• Monthly review of HIV indicators for retention • Liaise with Reg. HIV Coordinator to design and implement initiatives to increase retention at the service delivery level according to the National Protocol
Adherence Counsellor &	Conduct adherence activities at the clinic level	• Administer Treatment Readiness Assessment Tool (TRAT)

Staff	Role	Responsibility
Community Health Aides		• Daily review of patients appointed for the following week/ day to conduct appointment reminders. • Conduct telehealth adherence consultations between appointments • Conduct Directly Observed Therapy DOTS for select patients
Community Facilitator	Facilitates linkage and retention of peers	• Provides support to newly diagnosed PLHIV up to viral suppression • Conducts telehealth check ins between appointments • Provides monthly updates on peers
Medical Social Workers, Medical Social-Case Managers	Supervises Adherence Counsellors	• Administer Treatment Readiness Assessment Tool (TRAT) and review possible barriers to care. • Address barriers identified, this may require referral to support services • Weekly/daily review of patients appointed for the following week/day to ensure appointment reminders were done • Conduct telehealth consultations between appointments • Conduct field visits for persons based on findings from TRAT • Monthly review of HIV indicators for retention • Liaise with Team members through team meetings to flag possible PLHIV that may be at risk of missing appointments and develop retention activities for persons in care • Conduct field visits for patients unreachable by telephone call

Staff	Role	Responsibility
Psychologist	Provides psychological services/ therapy for PLHIV	• Develop and provide psychosocial counselling and guidance services which assist PLHIV and their families • Provide communication and act as liaison to social agencies/ agents of the state
Treatment, Care & Support Officer	Coordinate site level activities for treatment care & support	• Document all retention data along with the TCS team • Prepare monthly reports with relevant HIV data on retention • Contribute to monthly/ quarterly HIV/STI/TB reports • Implement initiatives to increase retention in care at the service delivery level according to National Protocol • Develop site specific plans along with TCS team to improve HIV treatment outcomes
Contact Investigators	Conducts field case investigation to locate out of care persons living with HIV	• Conduct field visits for patients unreachable by the Medical Social Worker/ Medical Social Worker-Case Manager • Document all retention and recovery data along with the TCS team.

Table 9: Roles and Responsibilities of Regional Staff for the Retention and Recovery Protocol

Staff	Roles	Responsibility
Regional Technical Director	Manages the HIV programme in the region	• Quarterly review of HIV indicators for retention in HIV care. • Ensure integration of HIV services within the Primary Health Care setting

Staff	Roles	Responsibility
HIV/STI/TB Coordinator	Coordinates HIV programme between the Parishes in the designated RHA	• Monthly review of HIV indicators - retention in care • Coordinate with other HIV Coordinators in other RHAs of shared cases retained in care • Design initiatives to improve retention in care at the service delivery level according to National Protocol • Report quarterly on retention in care through the quarterly HIV/STI/TB report
Regional Social Worker	Coordinates the Social Services and Case Management programme for the region	• Conducts monthly meetings with Psychosocial Team to ensure the implementation of activities pertaining to adherence and retention • Develops initiatives alongside the HIV Coordinator to improve retention in the RHA. • Liaises with Regional TCSO to develop pointed strategies for retention in care for key sites
Regional Treatment Care & Support Officer	Coordinate regional activities for treatment care & support	• Review retention data along with the Parish TCSOs • Prepares monthly reports with relevant HIV retention data • Implement initiatives to increase retention in care at the service delivery level according to National Protocol in conjunction with the regional SW and CI. • Develop regional plans along with Regional team to improve retention in HIV care

Table 10: Roles and Responsibilities of National Staff for the Retention and Recovery Protocol

Staff	Roles	Responsibility
Monitoring and Evaluation Officer	Provide technical assistance in monitoring and evaluation and support the regular analysis of datasets. Conducts data analysis and data quality verifications for validity, reliability and accuracy of databases used.	• Quarterly reports on indicators to monitor progress of retention. • Data verification of the national HIV treatment database for valid and reliable information on persons retained in care and on ARVs. • Provide technical assistance in data collection and analysis.
HIV/STI Information Officer	Establish and maintain the national HIV treatment database.	• Manage the national HIV treatment database and aid field staff in the use of the database for daily activities with regards to ensuring all data is present and correct for retention.
Director Treatment Care & Support	Manages the development, implementation, monitoring and evaluation of national policies and protocols for HIV, STI & TB	• Monthly review of the national picture as it pertains to retention in care and persons out of care. • Provide feedback based on reports, QA activities and implementation activities for retention in care. • Updates protocols and policies in keeping with international best practices and country context to improve retention in HIV care
Programme Development Officer HIV	Coordinates the development, implementation, monitoring and evaluation of national policies and protocols for HIV	• Reviews and provides feedback on monthly and quarterly regional reports on retention • Conducts quality assurance activities for HIV for retention

Staff	Roles	Responsibility
		• Reviews and provides feedback on initiatives to increase retention in care at the service delivery level according to National Protocol • Develops training programmes in keeping with international standards for HIV for retention in care
Programme Officer HIV	Coordinates the development, implementation, monitoring, and evaluation of national policies and protocols for Psychosocial Team	• Conducts quality assurance activities for psychosocial services - ensuring that services are provided to maintain retention in care • Reviews and provides feedback on monthly and quarterly Psychosocial reports for retention • Provides feedback on initiatives to increase retention in care at the service delivery level according to the National Protocol • Review site specific plans along with TCS team to improve HIV treatment outcomes at treatment sites. • Develops training programmes in keeping with international standards for psychosocial care facilitating retention in HIV treatment.
Programme Officer HIV (National TCSO)	Monitors HIV treatment implementation	• Reviews and provides feedback on monthly and quarterly TCSO reports for retention • Provides feedback on initiatives to increase retention in care at the service delivery level according to National Protocol • Reviews site specific plans along with TCS team to improve HIV treatment outcomes at treatment sites.

Retention and Recovery Protocol Process

It is intended that the Retention and Recovery protocol will provide the standard operating procedures in retaining PLHIV in care, and returning to care, persons living with HIV enrolled in care in the public and private treatment sites of the National HIV programme. The protocol should be conducted on all PLHIV enrolled in care and is divided into four processes:

Process 1: Pre-Appointment Process - prepare persons for upcoming appointments

Process 2: Attendance to specified appointment. (clinical, psychosocial (inclusive of support groups), pharmacy & laboratory)

Process 3: Retention Process - reschedule all missed appointments within 1 month

Process 4: Recovery Process - return to care persons designated as out-of-care

Preparation

1. In high volume and/or resource limited settings, staff members and responsibilities may deviate from the outline above in staff roles and responsibilities. It is at the discretion of each clinical site to establish a cadre of workers with clear responsibilities, as well as reporting and monitoring structures, to implement patient retention and recovery processes in coordination with regional and Ministry of Health counterparts.
2. Staff members who do not routinely make telephone calls for patient tracing are to consult with the Contact Investigators and Social Workers for advice on best practices. In addition, they are to complete the HIV Telemedicine training modules as outlined in the HIV Telemedicine Standard Operating Procedures.

Programme Tools

1. **Patient Contacting Script:** The script is an optional reference tool that can be used as a guideline for contacting patients through telephone calls and field visits.
2. **Patient Contacting Log-Sheet:** The log-sheet is used to record all patient contact attempts, including telephone calls and field visits, regardless of whether contact is successful.
3. **Patient Contacting Tracking Tool:** An electronic repository for information gathered from the Patient Contacting Log-Sheet.

4. **Out of Care Patient List:** The list is generated by the Treatment Services Information System (TSIS) that includes information for patients defined as "out of care" who are to be re-engaged through the recovery protocol. A patient is considered "out of care" if they meet one of the following criteria:
    a. Missed Appointment: a patient linked to care (e.g. attended one or more clinic appointments since diagnosis) and subsequently has not been seen at the clinic within 30 days of a missed appointment;
    b. Defaulter: a patient not seen at the clinic within 60 days of a missed appointment; and
    c. Lost to Follow-Up (LTFU): a patient who has not accessed treatment and care at a treatment site within 90 days of a missed appointment.
5. **Patient Retention and Recovery Reference Guide:** The reference guide is a one-page, double sided, laminated summary and flowchart that outlines the Patient Retention Protocol for Missed Appointment Patients and the Patient Recovery Protocol for Out of Care Patients. The guide is to be stored in an accessible location for easy reference by staff members contacting patients.
6. **Appointment Reminder Log-Sheet:** The log-sheet is used daily to record all patients who received an appointment reminder phone call.
7. **Missed Appointment Log-Sheet:** The log-sheet is used daily to record all patients who missed their appointment.

Pre-Appointment Process

This process entails conducting wellness check-ins and other assessments prior to scheduled appointment and conducting reminder calls to prevent any potential missed appointment and reschedule appointments if needed. These activities aim to enhance patient engagement, improve appointment adherence, and create more efficient clinic operations. The process is outlined in the table below. Please refer to the HIV Telemedicine Standard Operating Procedures (2021) for script and procedure of wellness check ins.

Table 11: Description of Activities during the Pre-Appointment Process

Procedure	Responsible Officers	Description
Wellness check-in phone call with PLHIV	Medical Social Worker Medical Social Worker-Case Manager Psychologist Adherence Counselor	A phone call with a PLHIV who has an extended appointment interval OR a chronic defaulter. To check on overall well-being, adherence to ART and assess additional patient needs. Scheduled approximately halfway between regular appointments using the standardized script from the **HIV Telemedicine Standard Operating Procedures**.
Appointment reminder phone calls	Medical Social Worker Medical Social Worker-Case Manager Adherence Counselor	A phone call with a PLHIV with an upcoming appointment to remind the patient of their upcoming appointment date. It is recommended that patients be called one day before and the day of the scheduled appointment to ensure that the patient attends the appointment. However, patient characteristics may also determine how many days prior the call should be made and how many reminders they should receive. A rescheduled appointment date during this call can be used to update next appointment date in TSIS.
Automated SMS Appointment reminder	Data Entry Clerk Adherence Counsellor	An automated text message reminder sent to a PLHIV with an upcoming appointment to remind the patient of their upcoming appointment date. This is a built-in feature on TSIS, which requires the patient's consent for usage. Once the patient has consented to receive SMS reminders, he/she will receive a message "Please remember your check-up appointment on ____ date"

Digital Message Alerts

Digital message alerts are a fast and effective way to communicate with patients. Outlined below are the guidelines for the implementation of the automated text messaging reminder system.

1. All patients upon their initial visit are to be informed of the use of text message alerts by the site staff to remind patients of upcoming appointments and/or missed appointments.
2. For all subsequent visits, the screening nurse is to verify telephone numbers for all patients with a clinic appointment.
3. Patients are to be told they have the option to opt-out of receiving text messages.
4. Patients opting-out are put into the "Do Not Text" group in the text messaging software via TSIS. Their telephone numbers are not recorded in the software.
5. Patients opting-in are put into the "Text" group in the text messaging software via TSIS. Their telephone numbers are recorded in the software.
6. Text messaging communication is not to be recorded as a contact attempt in the Patient Contacting Log-Sheet.
7. Messages are to be sent according to the following schedule:
   - One week before scheduled appointment;
   - Two days before scheduled appointment;

Appointment Process

The patient, after linkage to care, must be adherent to appointments and medication. Community Facilitators, Peer Navigators, or Adherence Counsellors monitor appointments scheduled for each day.

Patient encounters

1. The patient, based on the previous visit, will return to the clinic for specified appointment- the role of the retention navigator and or the community facilitator is essential in aiding the PLHIV to navigate the system according to the scheduled appointments
2. The patient at each clinical appointment is to be pre-assessed by a member of the psychosocial team or designate as outlined above so as to ensure that they attend the stated appointment in a seamless manner.
3. The patient is reassessed at the visit to determine any additional needs/ appointments as per existing protocols in the HIV Clinical Management Manual.

Table 12: Description of Activities during the Appointment Process for the Retention and Recovery Protocol

Procedure	Responsible Officers	Description
Registration	Health Records	Patient would have an appointment for the day and had medical record pre pulled for clinic visit.
Navigation of Health Facility	Adherence Counsellor Community Facilitator Peer Navigator	The PLHIV will be taken through the health facility to their designated appointment.
Clinic Visit	Social Worker Psychologist Adherence Counselor Case Manager Pharmacist Clinician Medical Technologist	Depending on the reason for visit, any of the following staff will be a part of the visit. There is execution of the visit as per standard SOPs and the HIV Clinical Management Manual. The patient is reappointed and followed up as per the pre-appointment process.

Retention Process

The patient retention process focuses on immediately re-engaging patients who have missed their scheduled appointment. It includes several components aimed at decreasing the number of missed appointments and ensuring patients receive the treatment, care, and support needed for the proper management of HIV.

Patient Retention Protocol for Missed Appointment Patients

1. Telephone numbers of patients are to be updated by the screening nurse at every visit.
2. Patients deemed at risk of defaulting are directed to an Adherence Counsellor at every visit. Patients most at risk are defined as any patient who is:
   a. In a treatment site for < 6 months;
   b. On ART <12 months;
   c. On ART >12 months and missed two appointments in the last 12 months.
3. The Community Facilitator, Peer Navigator, or Adherence Counsellor monitors appointments scheduled for each clinic day.
4. The Community Facilitator, Peer Navigator, or Adherence Counsellor is to call and send a text message reminder to patients who miss their appointment no later than one day after the missed appointment to schedule another appointment.
5. Patients who are not successfully contacted on the first telephone call attempt are to be called at least two more times, varying in days and times of day, within one week of the first call attempt.
6. If patients cannot be reached, next of kin should receive three additional telephone call attempts, if information is available.
7. All telephone call attempts and results are to be recorded in the **Patient Contacting Log-Sheet** by the staff member responsible for the contact attempt (Appendix C and D).

If Contact Is Made

1. Appointments are to be scheduled for the next available date with a target of within one week of the missed appointment, if possible.
2. The scheduled appointment date is to be recorded in the **Patient Contacting Log-Sheet.**
3. The day before and the day of the scheduled appointment, the Adherence Counsellor is to follow-up with a telephone call to ensure the patient is visiting the clinic. In addition, text message reminders are to be sent one week and two days prior to the appointment.
   a. Reminder calls and text messages are not recorded in the Patient Contacting Log-Sheet.

4. If patients do not attend rescheduled appointment, patients are referred to Lead Contact Investigator or Medical Social Worker.

If Contact Is Not Made

1. If contact is not made after telephone call attempts, patients are to be referred to the Medical Social Worker for field visit. The Medical Social Worker is to conduct a field visit within two weeks of the missed appointment date, or one week after the unsuccessful telephone call attempts.
2. If patients are not successfully contacted (but are determined reachable), the Patient Recovery Protocol for Out of Care Patients should be initiated. The process is managed by the Contact Investigators. Patients who are not successfully reached through field visit (but are determined reachable) are to be referred to a Lead Contact Investigator, who assigns the patients to the Contact Investigators responsible for the health district/zone in which the patient has a last known address.
3. All field visit attempts and results are to be recorded in the **Patient Contacting Log-Sheet** by the staff member responsible for the contact attempt.

Upon Patients Returning To Clinic

1. Community Facilitator, Peer Navigator, or Adherence Counsellor is to assist in the navigation of patients accessing care at the facility.
2. A Treatment Readiness Assessment Tool (TRAT) is to be completed for all returning patients to identify possible barriers to care.

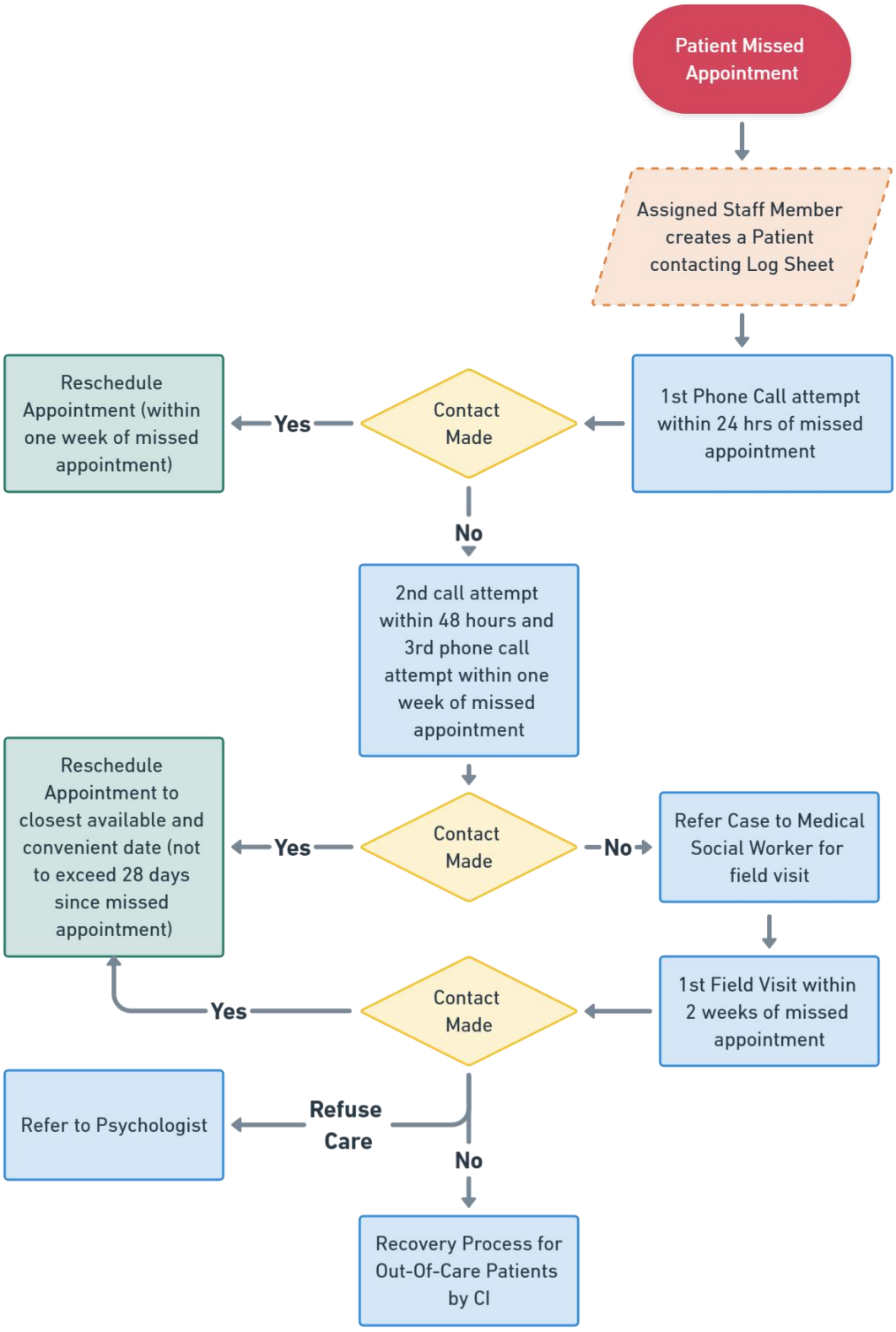


Figure 4: Patient Retention Protocol for Missed Appointment Patients

Recovery Process

The patient recovery process focuses on re-engaging patients who are considered out of care (those who have missed appointments and were not successfully contacted in the retention process, defaulters, and patients who are LTFU) to ensure proper management of HIV care.

Patient Recovery Protocol for Out of Care Patients

1. The Data Manager, Data Entry Clerk, or Treatment, Care and Support Officer (TCSO) is responsible for generating the Out of Care Patient List on the first working day of every month. *If necessary, the list is cleaned prior to distribution to prevent redundancies.*
2. The Community Facilitator, Peer Navigator, Adherence Counsellor, Case Manager or Social Worker is to call all patient defaulters, starting with those who have defaulted most recently.
3. Patients who are not successfully contacted on the first telephone call attempt are to be called at least two more times, varying in days and times of day, within the first two weeks of the month or until the patient is reached.
4. In the event that the team member(s) is referred to additional contacts through the next of kin, attempts are to be made to contact these persons a minimum of three times or until the person is reached.
5. All telephone call attempts and results are to be recorded in the **Patient Contacting Log-Sheet** by the staff member responsible for the contact attempt.

If Contact Is Not Made

1. If contact is not made after telephone call attempts, patients are to be referred to a Lead Contact Investigator, who assigns the patients to the Contact Investigators responsible for the health district/zone in which the patient has a last known address.
2. The assigned Contact Investigator is to conduct up to two field visits per patient, if they are determined to be locatable and require additional support. The first field visit is to be completed no later than one month from the date the Out of Care Patient List is generated and the second field visit completed no later than two months after.
3. If patients are not reached during field visits (recovery process – Fig.5), attempts should be made by the Contact Investigator to locate the person through local hospital information and review of the death registry. This step should be repeated on a quarterly basis until the criteria for case archiving has been met to conclude the case.

4. If patients are reached and do not attend their scheduled appointments, they are to be referred to the Psychologist for follow-up in the 3rd month.
5. All field visit attempts and results are to be recorded in the **Patient Contacting Log-Sheet** by the staff member responsible for the contact attempt.

If Contact Is Made With Next of Kin Only

1. If information is received regarding a potential patient death, migration off the island, or treatment in the private sector, record the results on the **Patient Contacting Log-Sheet**.
2. Otherwise, record that contact is made with next of kin on the **Patient Contacting Log-Sheet** for use during additional tracing efforts.

If Contact Is Made and Patients Are In Care Elsewhere

1. If patients indicate they are currently receiving treatment and care elsewhere, record the place of treatment.
    a. If the new place of treatment is a public site, the Data Manager, Data Entry Clerk, or Adherence Counsellor confirms the date of last appointment at the other site in TSIS. If not in TSIS, the Psychologist is to call the clinic for direct confirmation.
    b. If the other site has no record of the patients, ensure the patients return to the nearest treatment site at a specific date and time for a visit to validate the information given:
2. Inform the patients that a member of the treatment staff will be following up to confirm the information given;
    a. Provide the member of the treatment staff the patients' names, contact information, and scheduled return dates for follow-up.
3. Update the **Patient Contacting Log-Sheet** and TSIS database to reflect that the patients have a confirmed transferred.

If Contact Is Made and Patients Are Not Receptive

1. Patients are still encouraged to visit a treatment site.
2. Record patients' responses in **Patient Contacting Log-Sheet**.
3. Patients' names and contact information are to be sent to the Social Worker or Psychologist for follow-up.

4. The Medical Social Worker or Psychologist is to fill out the **Patient Contacting Log-Sheet** to indicate the results of contact attempts. If contact is made, complete the National Lost to Follow-Up Tracking Form.

If Contact Is Made and Patients Are Receptive

1. Patients are to be referred to the nearest treatment site, or treatment site that they are most comfortable attending, for the next available appointment with a target of no more than two weeks after contact.
2. The patient's planned return date is recorded on the **Patient Contacting Log-Sheet**.
3. Patients are to be informed that the Adherence Counsellor will be following up with them to ensure that they attend clinic on their scheduled date.
4. The patients' names and contact information are to be given to the Adherence Counsellor.
5. The day before and the day of the scheduled appointment, the Adherence Counsellor is to follow-up with a telephone call to ensure the patient is visiting the clinic. In addition, text message reminders are to be sent one week and two days prior to the appointment.
    a. *Reminder calls and text messages are not recorded in the Patient Contacting Log-Sheet.*
6. If patients do not return to clinic for scheduled appointment, the Adherence Counsellor is to follow Patient Retention Protocol for Missed Appointments.

Upon Patients Returning to Clinic

1. Recovered patients who physically enter the site are to see a Contact Investigator at first availability.
2. Case Manager and/or Psychologist are to administer the **National Lost to Follow-up Tracking Form** (Appendix G) to understand the reasons why patients were lost to follow up, in addition to their regular duties associated with such cases.
3. Recovered patients must also visit the Adherence Counsellor the same day and a Treatment Readiness Assessment Tool (TRAT) is to be completed for all returning patients to identify possible barriers to care.
4. The Community Facilitator, Peer Navigator, Case Manager to Adherence Counsellor is to assist in the navigation of patients accessing care at the facility.

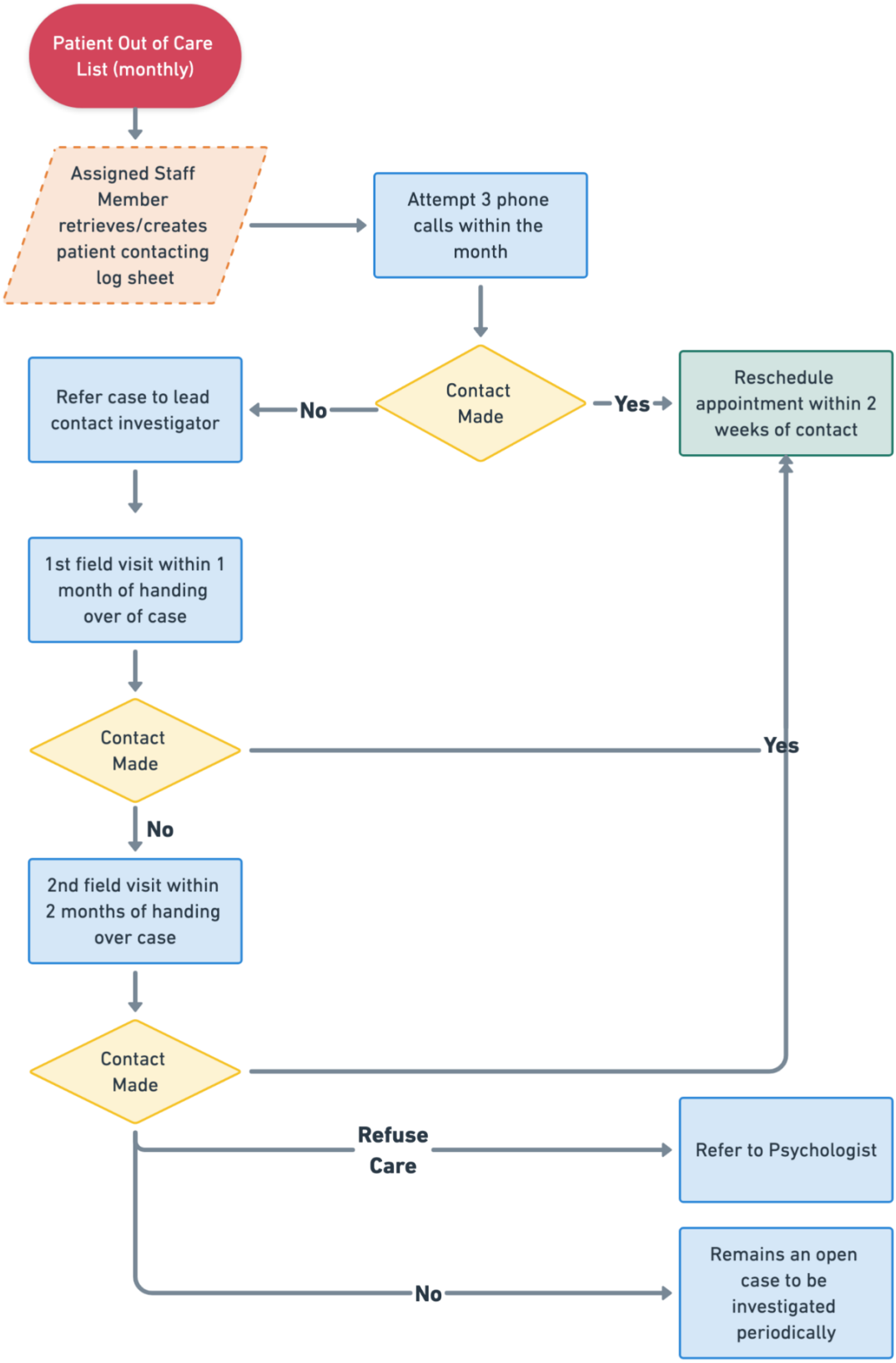


Figure 5: Patient Recovery Protocol for Out of Care Patients

Monitoring and Evaluation of the Retention and Recovery Protocol

The M&E for the Retention and Recovery Protocol, outlined below, should be incorporated in routine national HIV programme monitoring and evaluation.

1. On a daily basis the Adherence Counsellor, Case Manager, Community Health Aid or other assigned staff should complete a **Patient Contacting Log-Sheet** for each patient who missed an appointment. This **Patient Contacting Log-Sheet** should be used as the data source for the documentation of the retention and recovery process.
2. On a weekly basis the Data Entry Clerk, Data Manager, Adherence Counsellor, Treatment Care and Support Office or other assigned staff should record information from the **Patient Contacting Log-Sheets** into the **Patient Contacting Tracking Tool**. At this time the **Patient Contacting Log-Sheets** should be reviewed for completeness and any missing information added by the staff assigned to the case. **Patient Contacting Tracking Tool** can be used to collate data and report on the M&E indicators for the Retention and Recovery Process.
3. On a monthly basis the Social Worker/Case Manager or other assigned staff coordinating the retention and recovery process collates data from the **Patient Contacting Tracking Tool** and review data for completeness. This review is also to ensure that the patient has had the required amount of contact attempts.
4. **Patient Contacting Log-Sheets** are to be stored in a secured and locked cabinet accessible by staff assigned to the retention and recovery process. Sheets should be categorized as open or completed. **Patient Contacting Log-Sheets** are considered completed once the patient attends their rescheduled appointment date, confirmed deceased (refer to Resolution of Death Protocol), confirmed migrated, confirmed incarcerated and registered on TSIS or confirmed in care elsewhere and transfer completed on TSIS.
5. On a monthly basis the case manager, social worker or other assigned staff will collate information from the **National Lost to Follow-up Tracking Form** on the reasons PLHIV became lost to follow up. Noting the prevalence for the reasons for lost to follow up.
6. It is recommended that sites adopt a routine weekly retention and recovery meeting to review cases of PLHIV who missed appointment and are out of care to develop targeted intervention strategies to recover these persons and prevent recurrence of interruptions in care.

Table 13: Monitoring and Evaluation Indicators for Retention and Recovery Protocol

Indicator	Overview	Numerator	Denominator	Data Collection Methodology
Percentage of PLHIV with missed appointments	This indicator is key to measuring the effectiveness of the retention and recovery process. People who have missed appointments are those who do not visit the clinic on their recorded next appointment date.	The number of people who did not visit the clinic on their recorded next appointment date.	The number of persons with a next appointment date in the reporting month.	**The numerator** is generated by counting the number of cases recorded for the month in the Patient Contacting Tracking Tool of those persons who missed their appointment in the reporting month and had a Patient Contacting Log Sheet generated in the reporting month. **The denominator** is counted from TSIS of all persons with a next appointment date scheduled for the recording month.
Percentage of PLHIV with missed appointments rescheduled.	This indicator aims to measure the efficiency of contacting attempts in successfully reaching patients and having positive patient engagement with the rescheduling of the appointments.	The number of missed appointment cases that were reached and have a rescheduled appointment date.	The number of people who did not visit the clinic on their recorded next appointment date.	**The numerator** is generated by counting all PLHIV from the Patient Contacting Tracking Tool with successful contacting attempt recorded with a recorded rescheduled appointment date of persons who missed an appointment in the reporting month. **The**

Indicator	Overview	Numerator	Denominator	Data Collection Methodology
				denominator is generated by counting the number of cases recorded for the month in the Patient Contacting Tracking Tool of those persons who missed their appointment in the reporting month and had a Patient Contacting Log Sheet generated in the reporting month.
Percentage of PLHIV retained on ART	This indicator measures the proportion of PLHIV registered at the health facility who are retained on antiretroviral therapy (ART) providing a retention rate for the clinic.	The number of PLHIV with an active prescription (prescription end date in the future) OR a prescription end date no more than 28 days past.	The number of PLHIV registered in the health facility.	**The numerator** is generated by counting the number of persons with a prescription end date in the future or with a prescription end date no more than 28 days past as recorded in TSIS. **The denominator** is generated by counting all PLHIV on the health facilities register from TSIS.
Percentage of PLHIV who attended rescheduled	This indicator measures the effectiveness of patient engagement in the retention and recovery process that	The number of PLHIV who attended their rescheduled appointment	The number of missed appointment cases that were reached and	**The numerator** is generated by counting the number of patients who attended their rescheduled appointment

Indicator	Overview	Numerator	Denominator	Data Collection Methodology
appointment date.	can encourage patients to attend and continue care.	date after missing their appointment in a given month.	have a rescheduled appointment date.	dates within 28 days of missing their appointment. **The denominator** is generated by counting all PLHIV from the Patient Contacting Tracking Tool with successful contacting attempt recorded with a recorded rescheduled appointment date of persons who missed an appointment in the reporting month.

Data Sources

1. Patient Contacting Log Sheet
2. Patient Contacting Tracking Tool
3. TSIS 2.0

Frequency and Responsibility of Reporting

Data should be collected continuously at the parish level and aggregated quarterly. Data collection is to be carried out by the lead social worker or other assigned staff to coordinate the retention and recovery process at the site level and data reviews are the responsibility of the Parish Medical Officer of Health (MO(H)). The Regional HIV/STI/TB Coordinator will report on the indicators quarterly to the national level.

Appendix A: Laboratory Data Collection Tool (Entry to Care Protocol)

Date (dd/mm/yyyy)	**First Name**	**Last Name**	**Age**	**Date of Birth** (dd/mm/yyyy)	**Sex**	**Parish of Residence**	**Testing Site**	**Date of confirmatory Test** (dd/mm/yyyy)	**Notification Form** (Yes/No)	**Remarks**

Appendix B: Line Listing of PLHIV not Linked to Care (Entry to Care Protocol)

First Name	Last Name	Date of Birth	Sex at Birth	Address	Phone Number	Date of Confirmatory Test	Sufficient Identifying information	Investigation Start Date	Investigation End Date	Located (Yes/No)	Disposition (select a single disposition per case)

Alive and Linked
Alive and Not Linked
Migrated
Confirmed Death
Unable to Locate
Unlocatable due to insufficient identifying information
Presumed Death

Appendix C: Patient Contacting Log-Sheet Guidelines

1. All persons making telephone calls are responsible for filling out the **Patient Contacting Log-Sheet** as they conduct contact attempts. All persons contacting patients through field visits are responsible for completing the **Patient Contacting Log-Sheets** after each attempt. In both cases, the **Patient Contacting Log-Sheet** is to be completed regardless of the outcome.
2. A member of staff at each site is responsible for monitoring the **Patient Contacting Log Sheets** to ensure telephone calls and field outreach is conducted and the **Patient Contacting Log-Sheets** are completed correctly. It is recommended that the lead social worker be assigned this responsibility.
3. **Patient Contacting Log-Sheets** are to be kept in a secure and locked location, under the direct supervision of a member of the treatment site staff.
4. On a daily basis the Adherence Counsellor, Case Manager, Community Health Aid or other assigned staff should complete a **Patient Contacting Log-Sheet** for each patient who missed an appointment. This **Patient Contacting Log-Sheet** should be used as the data source for the documentation of the retention and recovery process.
5. On a weekly basis the Data Entry Clerk, Data Manager, Adherence Counsellor, Treatment Care and Support Office or other assigned staff should record information from the **Patient Contacting Log-Sheets** into the **Patient Contacting Tracking Tool**. At this time the **Patient Contacting Log-Sheets** should be reviewed for completeness and any missing information added by the staff assigned to the case. The **Patient Contacting Tracking Tool** can be used to collate data and report on the M&E indicators for the Retention and Recovery Process.
6. On a monthly basis, the Social Worker alongside the Treatment, Care and Support Officers process and collates data from the **Patient Contacting Tracking Tool** and review data for completeness. This review is also to ensure that the patient has had the required amount of contact attempts.

Appendix D: Patient Contacting Log-Sheet

Client Information					
Docket Number					
Parish of Residence					
Address					
Phone number(s)					
Classification	☐ LTFU	☐ Defaulter	☐ Missed Appointment		
Ever initiated ART	☐ Yes	☐ No			
Last Attended Clinic Appt: (DD/MM/YY) or NEVER	**Date of Missed Appt: (DD/MM/YY)**	**Date Patient Begins Recovery Process[1]: (DD/MM/YY)**			
Key Population	☐ PWID	☐ MSM	☐ TG	☐ FSW	☐ Prison, other	☐ None

Contact Attempts					
Contact Attempt: 1	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Contact Attempt: 2	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Contact Attempt: 3	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Contact Attempt: 4	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Contact Attempt: 5	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Contact Attempt: 6	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Contact Attempt: 7	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Contact Attempt: 8	**Staff Name:**	**Date:** **Time:**	☐ Phone Call ☐ Field Visit	**Contact Made** ☐ Yes ☐ No	**Date of Scheduled Appt:**
Notes:					

Patient Outcome			
☐ Incarcerated	☐ Migrated (out of country)	☐ Deceased	☐ Patient unreachable
☐ Refused to return to clinic	☐ In care elsewhere (private)	☐ In care elsewhere (public)	**New Clinic Name:**
Date Returned to Clinic: (DD/MM/YY)	**Date ART (re)initiated: (DD/MM/YY)**	**Confirmed on ARTs[2]:** ☐ Yes ☐ No	

Final Case Status		
☐ **Case closed[3]: (DD/MM/YY)**	☐ **Referred[4] to: (DD/MM/YY)**	☐ **Transferred: (DD/MM/YY)**

Data Entry			
Date Data Entered: (DD/MM/YY)	**Staff Initials for 1st Data Entry:**	**Date Data Double-Check: (DD/MM/YY)**	**Staff initials for Double Check:**

Last Updated: January 2019

[1] The date "Out of Care" list is generated, cleaned, and distributed to staff to begin patient recovery process

[2] Patient who reports filling ART prescription through another source with **absolutely no lapse in adherence** since last attended clinic appointment

[3] Defined as patients determined to be incarcerated, migrated, deceased or who have returned to clinic with a confirmed attended appointment

[4] Includes patients who return and are referred to treatment staff **AND** patients who refuse and are referred to Psychologist; include name of staff referral

Patient Contacting Log-Sheet Details/Code book

1. **Docket Number:** 6-digit unique identifier for patients; if docket number is not available, use KPAIDS number and note accordingly.
2. **Parish of Residence:** Parish in which patient resides, to be recorded from TSIS and confirmed by patient during contact. Parishes include: Clarendon, Hanover, Kingston, Manchester, Portland, St. Andrew, St. Ann, St. Catherine, St. Elizabeth, St. James, St. Mary, St. Thomas, Trelawny, and Westmoreland. Check one.
3. **Address:** Address in which patient resides, to be recorded from TSIS and confirmed by patient during contact.
4. **Phone number(s):** Contact telephone number for patient, to be recorded from TSIS and confirmed by patient during contact; can also include Next of Kin (NOK) numbers.
5. **Classification:** "Out of Care" patients classified in TSIS by the following definitions – LTFU: a patient who has not accessed treatment and care at a treatment site ≥ 90 days of missing a scheduled appointment; Defaulter: a patient not seen at the clinic within 45 – 89 days of a missed appointment; and Missed Appointment: a patient linked to care (e.g. attended one or more clinic appointments since diagnosis) and subsequently has not been seen at the clinic < 45 days of a missed appointment. Check one.
6. **Ever Initiated ART:** Patient has previously received a prescription for ARTs, as recorded in TSIS. Check Y/N.
7. **Last Attended Clinic Appointment:** Patient's last attended clinic appointment recorded in TSIS. Record date as DD/MM/YY or "NEVER" if no date is available.
8. **Date of Missed Appointment:** Date of the patient's missed appointment, which led to "Out of Care" classification. Record date as DD/MM/YY.
9. **Date Patient Begins Recovery Process:** Date "Out of Care" list is generated, cleaned, and distributed to staff to begin patient recovery process. Record date as DD/MM/YY.
10. **Key Population:** If patient identifies as a key population, check one: PWID (people who inject drugs), MSM (men who have sex with men), TG (transgender people), FSW (female sex workers), or Prison, other (people in prison and other closed settings). If patient does not identify as a key population, check "none."
11. **Contact Attempt 1-8:** Record name of staff who makes each contact attempt; date (DD/MM/YY) and time (24:00) of attempt; check type of attempt as phone call or field visit; check if contact was successfully made with patient and patient outcome determined (Y/N); record date of scheduled appointment (DD/MM/YY).
12. **Notes:** Record relevant information about patient and conversation; include details of communication if contact was made with other, callbacks, and/or reasons for refusal to return to clinic.
13. **Patient Outcome** – Incarcerated, Migrated (out of country), Deceased, or Patient Unreachable: Select patient disposition confirmed through contact attempts. Disposition of "patient unreachable" defined as exhaustive attempts (three telephone calls, two field visits) that do not lead to successful contact.
14. **Patient Outcome** – Refused to return to clinic, In care elsewhere (private), In care elsewhere (public): Record reason for patient decision not to return to clinic in notes section. If patient is in care elsewhere, record the new clinic name.
15. **Date Returned to Clinic:** Confirmed date patient returned to clinic and is no longer considered "out of care." This date may not be the same as the scheduled appointment date. Record date as DD/MM/YY.
16. **Date ART (re)initiated:** Date when patient receives a new prescription for ARTs. This date may not be the same as the date returned to clinic. Record date as DD/MM/YY.

17. **Confirmed on ARTs:** Patient who reports filling ART prescription through another source with absolutely no lapse in adherence since last attended clinic appointment. Check Y/N.
18. **Final Case Status** – Case Closed: Select for patients determined to be incarcerated, migrated, deceased, or who have returned to clinic with a confirmed attended appointment. Record date as DD/MM/YY. Determined only by Lead CI.
19. **Final Case Status** – Referred to: Select for patients who return and are referred to treatment staff AND patients who refuse and are referred to Psychologist; include name of referral and record date as DD/MM/YY. Determined only by Lead CI.
20. **Final Case Status** – Transferred: Select for patients with a site transfer confirmed via TSIS or by contacting other clinic. Record date as DD/MM/YY. Determined only by Lead CI.
21. **Data Entry** – Date Data Entered, Staff Initials for 1st Data Entry, Date Data Double Check, Data Entry – Staff Initials for Double Check: Record dates (DD/MM/YY) and staff names to ensure accountable data entry.

Appendix E: Patient Contacting Script for the Retention and Recovery Protocol

The Patient Contacting Script is to be used as a guideline for all staff members who contact patients through the patient retention and recovery processes. This includes the Community Facilitator, Peer Navigator, Adherence Counsellor, Lead Contact Investigator, Contact Investigator, and any additional staff members involved in patient contacting and tracing.

The "Client Information" section of the Patient Contacting Log-sheet is to be completed prior to the first contact attempt. The "Contact Attempt" and "Patient Outcome" sections are to be completed during the phone call contact attempt or after the field visit contact attempt.

Instructions are underlined.

1. Greet the respondent: (e.g., "Good morning!"), May I speak with (patient's first name)?

1. A. Patient answers by first name	1. B. Someone else answers
Confirm patient's identity through any/all of the following questions: • Can you please remind me of your last name? • Can you please remind me of your pet name or alias? • Can you please remind me of your mother's maiden name? • Can you please remind me of when you were born? • Have you ever visited (clinic name)? When was your last visit there? If identity is confirmed, continue.	Verify if patient uses the phone or lives there. • Phone Call: Ok, who else uses your phone? • Field Visit: Ok, who else lives here?

If identity is NOT confirmed, seek additional information. Stop if needed.	
My name is _____ and I'm from the clinic.	• <u>Phone Call:</u> This a friend of (patient's first name). Is there another number available to reach him/her? Or is there a better time or another number to call back? • <u>Field Visit:</u> I am a friend of (patient's first name). Is there another place to reach him/her? Or is there a better time to come back?
□ Continue to 2.	• In the Patient Contacting Log-sheet, check contact made: "No." • Include contact made with other in "Notes" section and any additional information for follow up. **[END]**

2. Engage in friendly conversation with patient to build connection and establish trust. This step is especially important if you are not familiar with the patient.

3. I'd like to share some information with you and ask some basic questions. Everything we say will remain between you and me. Is now a good time for us to talk a bit more?

If the patient denies living with HIV, do not argue with him/her. Firmly and in a friendly manner, encourage them to come in for a confirmatory test. If they agree, skip to 7(i) to schedule and confirm appointment.

3. A. Yes	3. B. No
□ Continue to 4.	When would be a better time for me to call you? • In the Patient Contacting Log-Sheet, check contact made: "No," as additional contacts are required. • Write the time they indicate for a return call in the "notes" section. **[END]**

4. I noticed that we have not seen you in a while. Can you tell me where you are now living?

If conducting a field visit, confirm field visit location is home address.

Note the address on the Patient Contacting Log-sheet, if not already there or if different.

5. Are you still receiving care?

5. A. Receiving care somewhere else	5. B. Not receiving care somewhere else
• Check "In care elsewhere" and record new clinic name in the "Patient Outcome" section of the Patient Contacting Log-Sheet. • Continue to 8.	□ Continue to 6.

6. Have you ever taken HIV medication (or ARVs, antiretroviral drugs)?

6. A. Yes	6. B. No
□ Go to 7A (treatment experienced).	□ Go to 7B (pre-ART).

7. A. If treatment experienced and needs to return to the clinic, read text:

Here at (clinic name), we care about you and your health. It has been a while since we've seen you and we want to make sure you are continuing to take your HIV medication (ARVs) and are feeling well. It is very important to regularly take your HIV medication (ARVs) because it will keep you healthy, strong, and live longer. When you take it consistently, HIV medication (ARVs) can reduce the amount of virus in your body so that it can be undetectable. This means the virus cannot be passed to others (e.g., to your sexual partner or baby during pregnancy or breastfeeding). With HIV medication (ARVs), your CD4 count will remain high, which can help you stay healthy. But without HIV medication (ARVs), your CD4 count will continue to drop and your immune system will weaken, which will increase your risk of other infections.

We want to help you be strong and healthy, so we request that you visit our clinic as soon as possible. Then we can talk about this some more, you can receive any necessary tests, and restart your HIV medication (ARVs). Once you come in for your appointment, we can give you more information about the benefits of HIV medication (ARVs) and the options you have. We can also offer more information about additional services to meet your other health needs.

When would you be willing to come in? Our hours of operation are _____ (e.g., 9am to 5pm, Monday and Thursday and 5pm to 9pm on Thursday night).

Please follow the appropriate steps based on whether patient is receptive (7(i)) or not receptive (7(ii)) to returning to the clinic.

7. B. If pre-ART and needs to return to the clinic, read text:

Here at (clinic name), we care about you and your health and offer health services, such as receiving HIV medication (ARVs), to you at no cost. It is now recommended for all persons living with HIV, regardless of CD4 count or symptoms, to take HIV medication (ARVs). Persons with HIV should start treatment as early as possible because the sooner you start treatment, the healthier you will be and the less likely you

will be to develop other illnesses like TB, cancer, or kidney disease. Also, when you take it consistently, HIV medication (ARVs) can reduce the amount of virus in your body so that it can be undetectable. This means the virus cannot be passed to others (e.g., to your sexual partner or baby during pregnancy or breastfeeding). With HIV medication (ARVs), your CD4 count will remain high, which can help you stay healthy. But without HIV medication (ARVs), your CD4 count will continue to drop and your immune system will weaken, which will increase your risk of other infections.

We want to help you be strong and healthy, so we request that you visit our clinic as soon as possible. Then we can talk about this some more, you can receive any necessary tests, and start your HIV medication (ARVs). Once you come in for your appointment, we can give you more information about the benefits of HIV medication (ARVs) and the options you have. We can also offer more information about additional services to meet your other health needs.

When would you be willing to come in? Our hours of operation are _____ (e.g., 9am to 5pm, Monday and Thursday and 5pm to 9pm on Thursday night).

Please follow the appropriate steps based on whether patient is receptive (7(i)) or not receptive (7(ii)) to returning to the clinic.

7 (i). Patient is receptive:	7 (ii). Patient is not receptive:
Ask patient for preferred date. If the patient gives no preference, give the earliest appointment available based on the appointment logbook and timelines outlined in Ministry of Health's Patient Retention and Recovery Program Protocol.	Probe for reasons and provide the appropriate response based on patient's reasons for not wanting to visit the clinic. Continue until the patient agrees to return with a scheduled appointment. First ask a broad question, such as the following:

OK, I have confirmed your appointment. Do you have any other questions for me? We will wait for you at (clinic name) on (date and time). □ Record the scheduled date for the corresponding contact attempt on the Patient Contacting Log-Sheet. **[END]**	OK. Tell me—just between you and me—what are your hesitations or what's getting in the way? I'm on your side.
	If there is no response after 3 seconds, continue: • structural factors (e.g., distance and transportation) o Do you have difficulties getting to the clinic? • then clinic-based factors (e.g., wait time, provider attitude) o Have you had any unsatisfactory or unfriendly experiences at our clinic? • followed by psychosocial factors (stigma, depression) o What is else is there? Is there anything else on your mind? Remember, I'm on your side. • Write reasons patient is not willing or hesitant to come in the "Notes" section of the Patient Contacting Log-Sheet.

	If it is clear that the patient will not return to the clinic for fear of being seen by acquaintances: Would you be interested in attending another public or private clinic in the area? Share a pre-prepared list of alternative public and private clinics in the parish and the region. □ In the "Patient Outcome" section of the Patient Contacting Log Sheet, check the patient "refused to return to clinic" for further follow up. **[END]**
	If the patient is absolutely reluctant to return: Thank you for taking the time to speak with me. Before we hang up, I want you to know that when
	you are ready, you are always welcome back here at (clinic name)! □ In the "Patient Outcome" section of the Patient Contacting Log Sheet, check the patient "refused to return to clinic" for further follow up. **[END]**

8. May I ask where are you currently getting your HIV services?

In the “Patient Outcome” section of the Patient Contacting Log-Sheet, select if the patient is in care elsewhere in a public or private clinic and the new clinic name (or address of the clinic if you do not recognize the name).

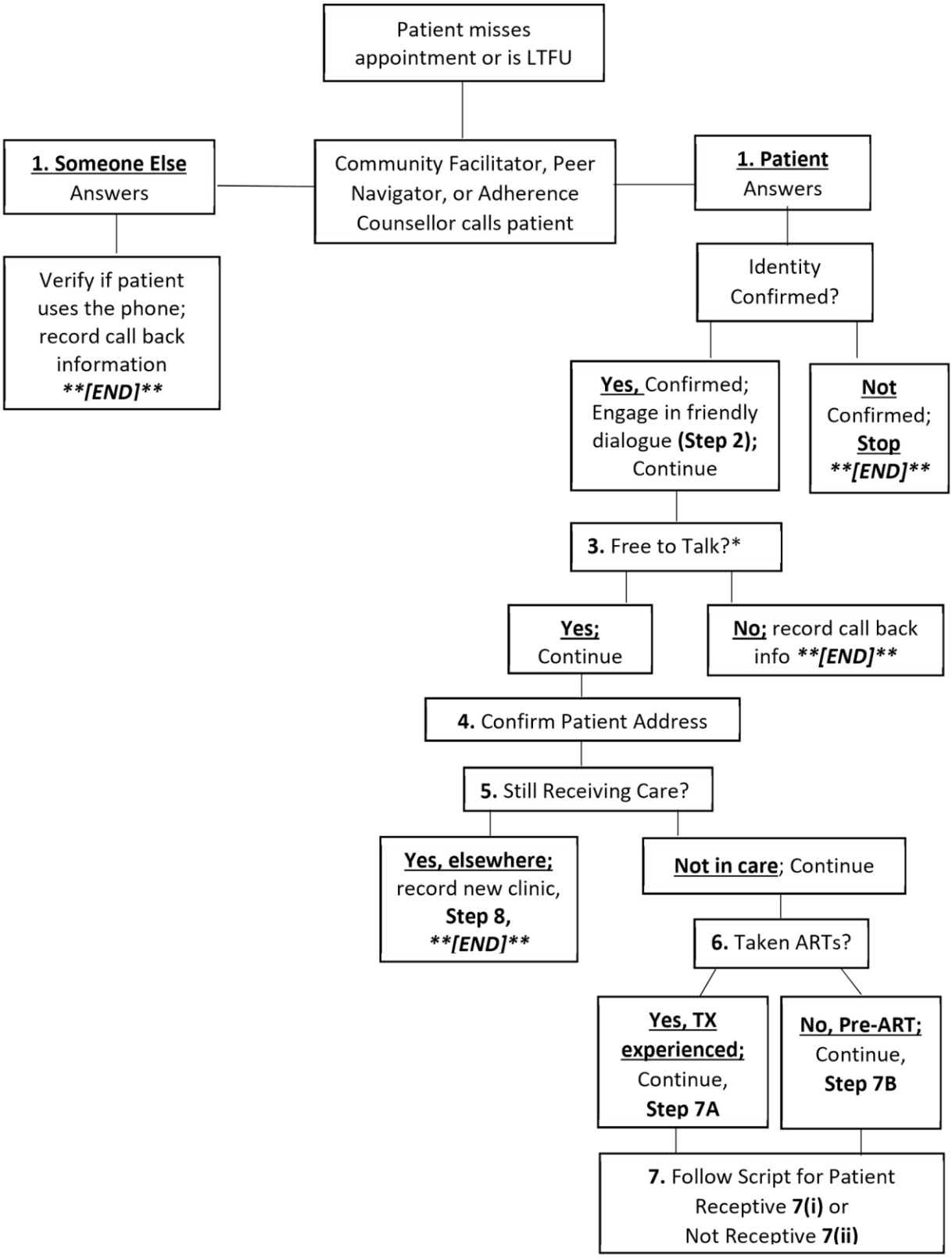
My colleagues at (new clinic name) and I care about you and your health, so we wanted to make sure you are able to get your HIV treatment (ARVs). It is important to continue HIV treatment (ARVs) as advised by your doctor and not to miss any doses of your medication, so that you can stay strong and healthy.

Do you have any questions or concerns for me?
Refer to standardized responses for structural, clinic-based, and psychosocial challenges in Appendix A.

Before we go, I want you to know that you are always welcome back here at (clinic name)!

** [END] **

Flow Chart: Patient Contacting Script



*If patient denies living with HIV, **do not** argue. Firmly encourage a confirmatory test.
If receptive, schedule and confirm appointment, **Step 7(i).**

STRUCTURAL	CLINIC-BASED	PSYCHOSOCIAL
"I don't have the money for this treatment." Here in Jamaica, any person living with HIV can receive medication at no cost to him/her. "I don't have a home/stable housing/food." Here at our clinic, we have a team of providers who can provide you with medication as well as connect to resources to help address your needs regarding housing/food. "I don't have the means of getting there." Or "Public transportation costs are too high for me." How much is it? Can you borrow the money from someone, mentioning that this is for something important (health matter)? "I'm physically not able to go." or "I need assistance to get to the clinic." Let me ask our physician how we can assist you. We'll call you back. "My work and family commitments keep me from taking time out of my schedule."	"I have bad side effects from the medication." Here at our clinic, one of our providers can prescribe a different type of medication that you might tolerate better. We will work with you until you find something that works for you. "I don't feel welcomed there." Or "I felt/was mistreated by clinic staff." I apologize on behalf of our clinic about your experience there. Know that we strive to improve the quality of our services through training and education to make sure we deliver friendly and high-quality care to all patients, no matter their HIV status and no matter who they are. "My work and family commitments keep me from taking time out of my schedule." Short-term: Here at our clinic, we provide the option for you to come for a consultation _____ (e.g., on a Saturday or during the late evening hours of Thursday).	"I'm still not convinced that I need treatment." Starting medication as early as possible, before your immune system becomes weak, helps to lead to live a longer and healthier life. Medication reduces the chance of you getting serious infections like TB Medication decreases the risk of passing it on to others along with safer sex practices. "I am/feel healthy and strong." I'm glad. To keep you feeling that way, you need to keep a healthy level of CD4—starting medication early will help with that. "I don't have the energy to go." Let me ask our physician how we can assist you. We'll call you back. "I don't want anyone to find out." It is our job to keep matters about your health confidential and to protect your privacy. "I don't want to be seen by anyone else at the clinic."

Short-term: Here at our clinic, we provide the option for you to come for a consultation_____ (e.g., on a Saturday or during the late evening hours of Thursday.) Long-term: Once you're stable on medication, your provider can give you the option of getting up to three month's supply of medication. That way, you'll only come to the pharmacy every three months. "I'm just too busy and don't have a lot of time to spend at the clinic." Short-term: We will try to accommodate you as quick as possible. Long-term: Once you're stable on medication, your provider can give you the option of getting multiple months of medication. That way, you'll only come to the pharmacy every three months	Long-term: Once you're stable on ART, your provider can give you the option of getting multiple months of medication. That way, you'll only come to the pharmacy every three months. "I'm just too busy and don't have a lot of time to spend at the clinic." Short-term: We will try to accommodate you as quick as possible. Long-term: Once you're stable on medication, your provider can give you the option of getting up to three month's supply of medicine. That way, you'll only come to the pharmacy every three months.	We will try to accommodate you as quick as possible, so that you are in and out in as little time. "I know the workers/people who attend that clinic" It is our job to keep matters about your health confidential and to protect your privacy. "As an LGBT person, I don't know if I'll be welcomed." I'm on your side. Know that we strive to improve the quality of our services through training and education to make sure we deliver care to all patients, no matter who you are. "As a sex worker, I don't know if I'll be welcomed." I'm on your side. Know that we strive to improve the quality of our services through training and education to make sure we deliver care to all patients, no matter who you are or what you do.

Appendix F: Patient Contacting Tracking Tool

Docket No.	Parish of Residence	Address	Contact #	Classification	Ever Initiated ART

Last Attended Clinic Appt Date	Date of Missed Appt	Date Patient Begins Recovery Process	Key Population

Contact #1				Contact #2			
Date	Type	Status	Scheduled Appt Date	Date	Type	Status	Scheduled Appt Date

Contact #3				Contact #4			
Date	Type	Status	Scheduled Appt Date	Date	Type	Status	Scheduled Appt Date

Contact #5				Contact #6			
Date	Type	Status	Scheduled Appt Date	Date	Type	Status	Scheduled Appt Date

Contact #7				Contact #8			
Date	Type	Status	Scheduled Appt Date	Date	Type	Status	Scheduled Appt Date

Notes	Patient Outcome	New Clinic Name	Date Returned to Clinic	Date ART	Confirmed on ART	Final Case Status	Date of Final Case Status

Legend for Patient Contacting Tracking Tool						
Classification	**Yes/ No**	**Attempt Type**	**Patient Outcome**	**Final Case Status**	**Parish of Residence**	**Key Population**
LTFU	Yes	Phone Call	Incarcerated	Case Closed	Hanover	PWID
Defaulter	No	Field Visit	Migrated (Out of Country)	Referred to	St. Elizabeth	MSM
Missed Appointment			Deceased	Transferred	St. James	TG
			Patient Unreachable		Trelawny	FSW
			Refused to Return to Clinic		Westmoreland	Prison, other
			In Care Elsewhere (Private)		Clarendon	None
			In Care Elsewhere (Public)		Manchester	
			Returned to Care		St. Ann	
					St. Catherine	
					St. Mary	
					Kingston	
					Portland	
					St. Andrew	
					St Thomas	

Appendix G: National Lost To Follow-Up Tracking Form

LOST TO FOLLOW-UP TRACKING FORM

SECTION A: ADMINISTRATIVE

A1. Patient name:		A2. Patient Record Number	
A3. Clinic name:		A4. ART Status Of Patient	☐ Not yet started ART ☐ Has started ART
A5. Date of initial enrollment in clinic:	DAY MONTH YEAR	A6. Date last seen in clinic:	DAY MONTH YEAR

SECTION B: TRACKER COMMUNICATES WITH PATIENT

B1. Interview date. DAY MONTH YEAR

B2. Have you seen a doctor or nurse for the care or treatment of HIV within the last three months?

○Yes

→B2a. Where? *Mark all that apply.*
☐ Original clinic ☐ Different clinic
☐ Private physician ☐ Other, specify:

→B2b. When did you first make a visit to **any** new clinic? DAY MONTH YEAR

→B2c. Why are you going to [new clinic] instead of your previous one? *Mark all that apply.*

Access to Care
☐ Transportation is easier or cheaper to new clinic.
☐ I spend less money at new clinic.
☐ I spend less time at new clinic.
☐ Starting ART is easier new clinic.
☐ Treatment supporter not required at new clinic.
☐ Fewer administrative requirements at new clinic.

Work and Family
☐ New clinic is closer to work.
☐ I had family obligations.
☐ Family conflict prevented attending clinic.
☐ My HIV status was less likely to be discovered by my family at new clinic.

Clinic Factor
☐ Old clinic ran out of medications ("stock-out").
☐ The staff is nicer at new clinic.
☐ Care is better at new clinic.
☐ The waiting area is more comfortable at new clinic.
☐ My HIV status is less likely to be discovered by people I know at new clinic.
☐ I was afraid old clinic would scold me for missing appointment.
☐ I received more goods (i.e., food, bednets) at new clinic

Other
☐ Declines to answer
☐ Other, specify:

○No

→B2d. Why did you stop going to **any** clinic for your HIV care? *Mark all that apply.*

Access to Care
☐Transportation was too difficult or expensive.
☐ I didn't have enough money to access care.
☐ I spent too much time at clinic.
☐ It was taking too long to start ART.
☐ I couldn't find a treatment supporter that was required.
☐ Too many administrative requirements.

Work and Family
☐ Work interfered with picking up medications or visiting clinic for review.
☐ Work interfered with taking medications.
☐ I had family obligations.
☐ Family conflict prevented attending clinic.
☐ Attending clinic risked disclosure to my family that I had HIV.

Medical
☐ I felt too sick to come to clinic.
☐ The medicine was not helping me feel better.
☐ I was experiencing side effects from the medicine.
☐ I felt well and thought I didn't need care.
☐ I didn't want to take drugs forever.
☐ I was taking too many pills a day.
☐ I didn't have enough food.
☐ I was drinking alcohol.

Clinic Factor
☐ The clinic ran out of medications ("stock-out").
☐ The staff was not nice.
☐ The care was not good.
☐ The waiting area was not comfortable.
☐ Attending clinic risked disclosure that I had HIV.
☐ I was afraid clinic would scold me for missing my appointment.
☐ I stopped receiving goods (food, stipend, etc.) at clinic.

Alternative Treatment and Advice
☐ A family member or other important person told me to stop going to clinic.
☐ My doctor or nurse told me to stop going to clinic.
☐ Because I went to someone who tried / is trying to cure me by prayer / religious rituals.
☐ Because I saw / am seeing a traditional healer instead

Other
☐ Declines to answer
☐ Other, specify:

B3. Have you taken antiretroviral medicine (defined as any ART) in the last 14 days?

○Never Started

○Yes

→ B3a. Where did you get the medicine? *Mark all that apply*
☐ Original clinic ☐ Friend or family member
☐ Private physician ☐ Other, specify:

→ B3b. Can you show me or name your medicines? *If phone interview ask "Can you name your medicines?"*
○ Yes, → *Record the medicines*
○ No, patient unable to name or show medicines
1. 2. 3. 4.

○No

→ B3c. When did you stop ART? DAY MONTH YEAR

→ B3d. Why did you stop ART? *Answer both coded and descriptive responses*

Access to Care
☐ Transportation was too difficult or expensive.
☐ I didn't have enough money to access care.
☐ I spent too much time at clinic.
☐ Too many administrative requirements.

Work and Family
☐ Work interfered with picking up medications or visiting clinic for review.
☐ Work interfered with taking medications.
☐ I had family obligations.
☐ Family conflict prevented attending clinic.
☐ Attending clinic risked disclosure to my family that I had HIV.

Medical
☐ I felt too sick to take the medicines.
☐ The medicine was not helping me feel better.
☐ I was experiencing side effects from the medicine.
☐ I felt too sick to come to clinic.
☐ I felt well and thought I didn't need care.
☐ I didn't want to take drugs forever.
☐ I was taking too many pills a day.
☐ I didn't have enough food.
☐ I was drinking alcohol.

Clinic Factor
☐ The clinic ran out of medications ("stock-out").
☐ The staff was not nice.
☐ The care was not good.
☐ The waiting area was not comfortable.
☐ Attending clinic risked disclosure that I had HIV.
☐ I stopped receiving goods (i.e food, bednets) at clinic.

Alternative Treatment and Advice
☐ A family member or other important person told me to stop taking ART.
☐ My doctor or nurse told me to stop taking ART.
☐ Because I went to someone who tried / is trying to cure me by prayer / religious rituals.
☐ Because I saw / am seeing a traditional healer instead.

Other:
☐ Declines to answer
☐ Other, specify:

Appendix H: Job Aid 1 – Linkage and Retention Standards of Service Delivery

Linkage and Retention PROTOCOL

Standards of Service Delivery

LINKAGE LEVEL	TYPE OF SERVICE	RESPONSIBLE STAFF	MEASURE OF COMPLETION
IDENTIFICATION	- Post-Test Counselling - Generate Class 1 Notification	Tester, CI, other PTC (Government, NGOs, Private)	**Class 1 Notification reaches MOH**
PREPARATION	- Identify appropriate facility	CI, other PTC (Government, NGOs, Private)	**Patient linked with appropriate facility**
	- Referral if required	CI, other PTC (Govt., NGOs, Private)	**Patient reaches referral facility**
	- Same day appointment scheduling: for first doctor visit and CD4	Contact Investigator	**Patient gets appointment on the same day of testing**
	- Assessment for barriers to linkage (medical, social and emotional) of patient	Medical Social Worker, Medical Social Worker- Case Manager, Psychologist Adherence Counsellor	**Patient linked to social support services**
OUTCOME	- Linkage to treatment site (firstdoctor's visit and CD4)	Combined effort of all CI, MSW, MSW-CM, AC, and others	**Patient attends first doctor's visit at treatment site** **Patient does CD4**
FOLLOW-UP	- Monitoring	Contact Investigator	**Linkage measured using performance indicators**
	- Follow up with patient to ensurethat appointment is kept - Track Lost to Follow up	Adherence Counsellor	**Patient does not miss appointments**
	- Assist patient to adhere totreatment regime	Adherence Counsellor	**Patient adheres to treatment regime**
	Referral feedback and referraltracking	Contact Investigator,	**Outcome of referral is documented**

Appendix I: Job Aid 2 – Entry to Care Roles and Responsibilities

ENTRY TO CARE PROTOCOL

Roles and Responsibilities of Parish Staff

Medical Officer of Health

- Monthly review of HIV indicators for linkage and ETC

Community Peer Educators

- Link positives identified with contact investigator

BCC Officer

- Coordinate and implement prevention programme at the parish level

Community Facilitators

- Provides support to newly diagnosed PLHIV up to viral suppression.

Contact Investigator

- Laboratory Surveillance
- Contact Tracing
- Case Investigations
- Testing and Referrals
- Linkage to Care
- Documentation of all linkage and ETC data along with the TCS team

Treatment Care and Support Officer

- Document all linkage and ETC data along with the TCS team
- Prepare monthly reports with relevant HIV data on linkage
- Contribute to monthly/ quarterly HIV/STI/TB reports
- Implement initiatives to increase linkage to care at the service delivery level according to National Protocol

Appendix J: Job Aid 2 – Retention and Recovery Roles and Responsibilities

RETENTION & RECOVERY PROTOCOL

Roles and Responsibilities of Parish Staff

Medical Officer of Health

- Monthly review of HIV indicators for retention

Treatment Care and Support Officers

- Document all retention data along with the TCS team
- Generate out-of-care lists

Contact Investigator

- Conduct field visits for patients unreachable by telephone calls

Community Facilitators

- Conducts telehealth check-ins between appointments
- Provides monthly updates on peers

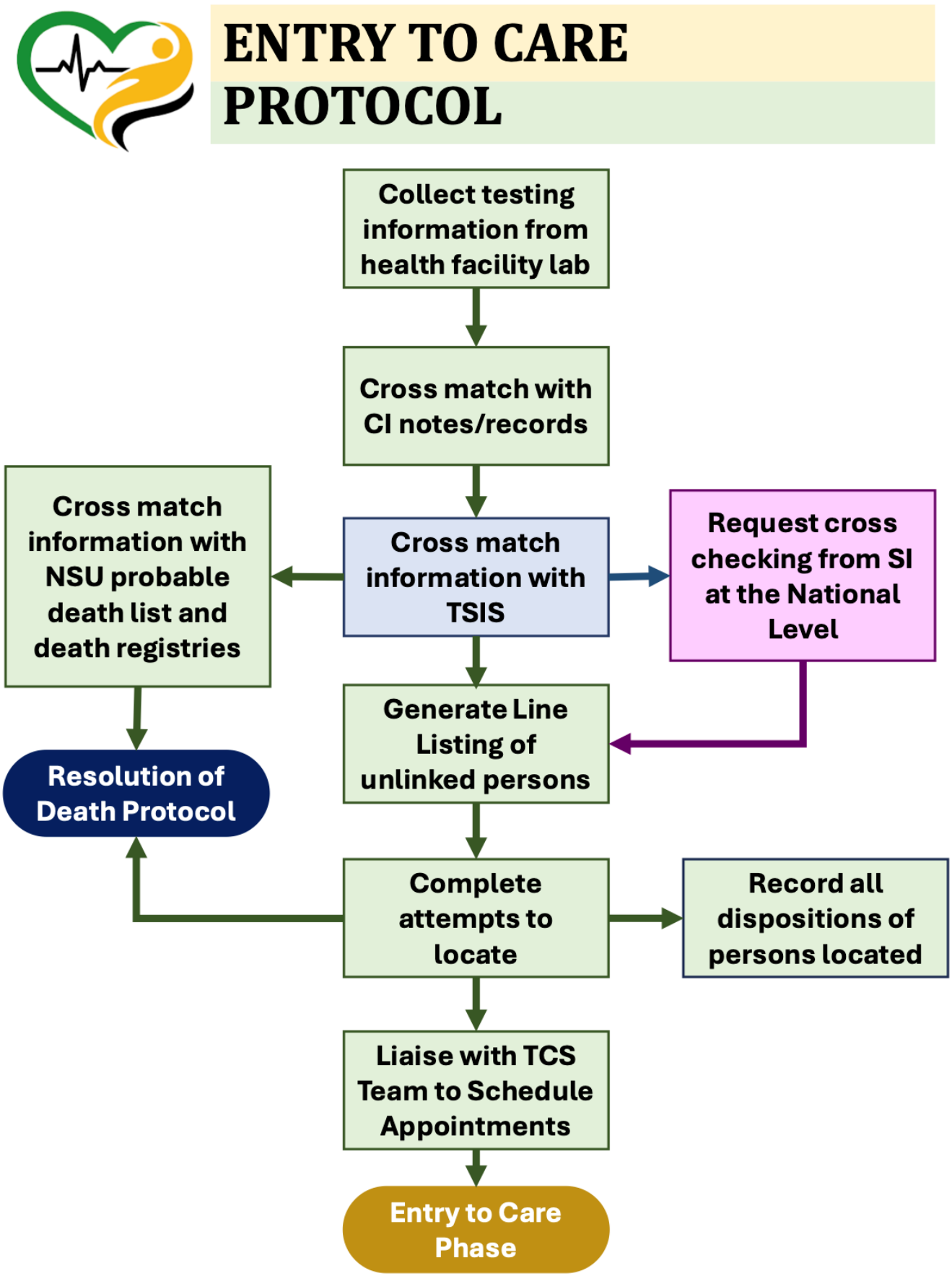
Social Worker & Case Managers

- Administer TRAT and review possible barriers to care
- Weekly/Daily review of patients appointed for the following day/week.
- Conduct telehealth consults
- Conduct field visits for persons based on findings from TRAT
- Weekly/Monthly Review of indicators for retention.

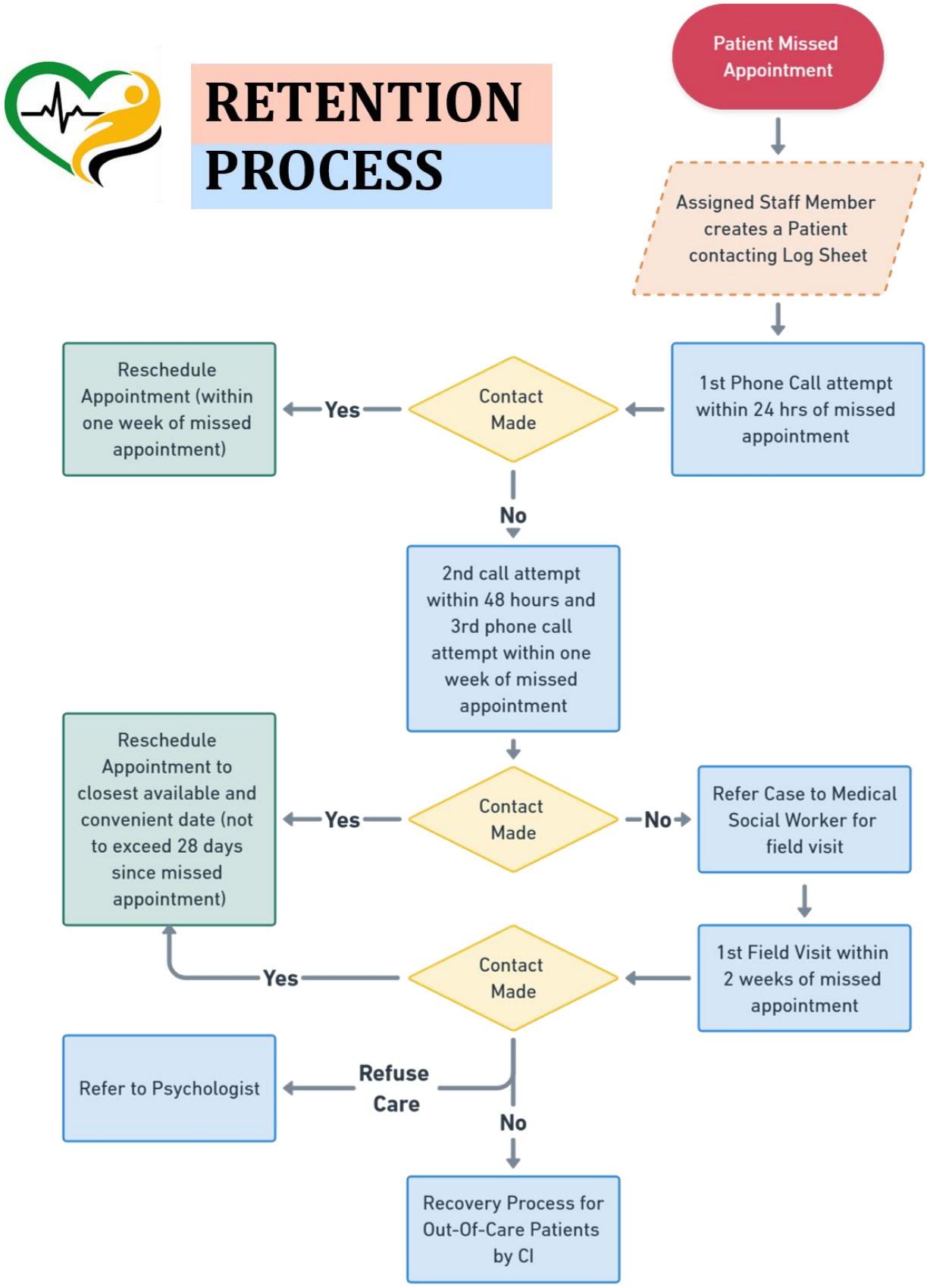
Adherence Counsellors & Community Health Aides

- Administer TRAT
- Daily review of patients appointed the following day to conduct appointment reminders
- Conduct telehealth adherence consultations
- Conduct Directly Observed Therapy for select patients
- Document all retention data along with the TCS team

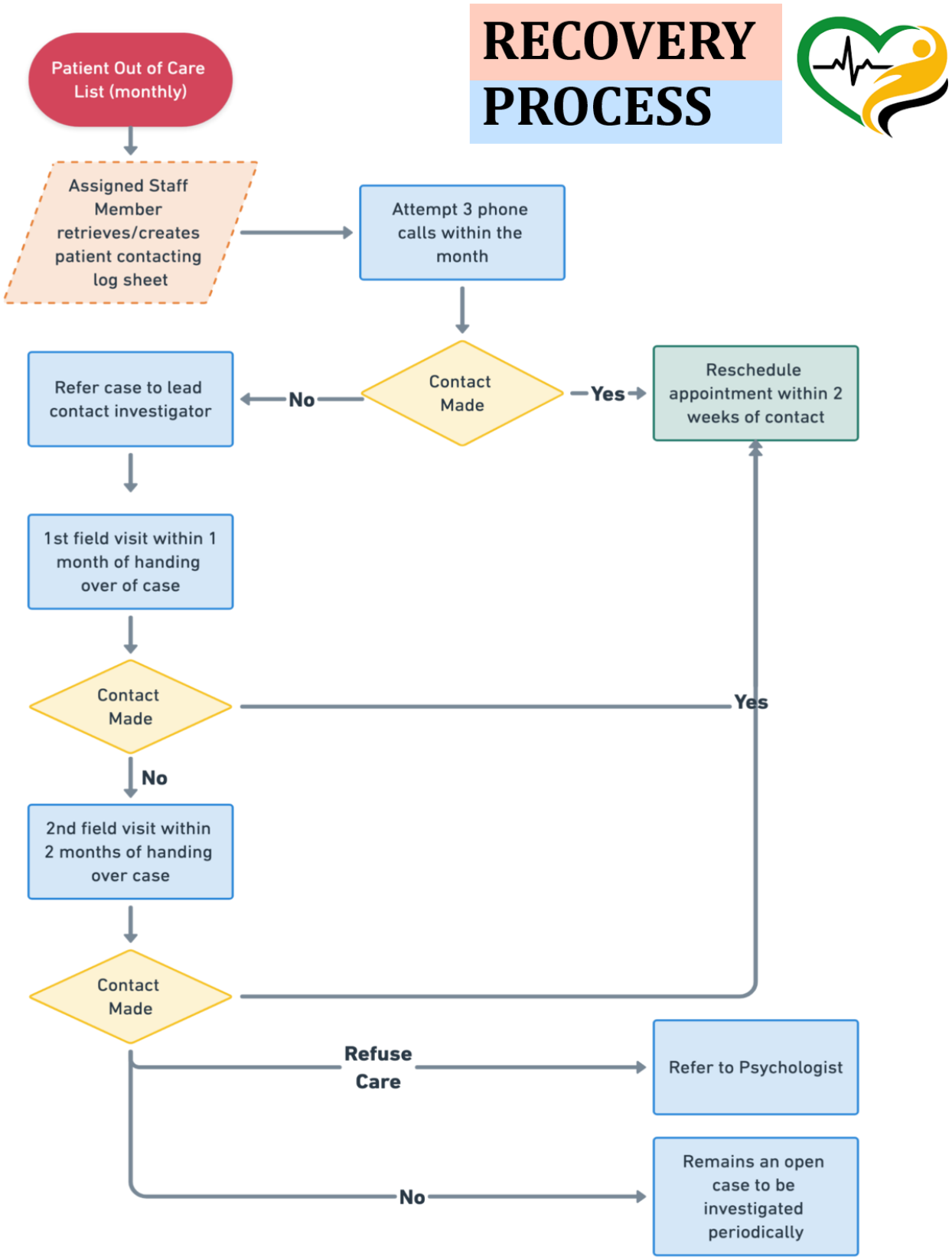
Appendix K: Job Aid 3 – Entry to Care Protocol Flowchart



Appendix L: Job Aid 4 – Retention Protocol Flowchart



Appendix M: Job Aid 5 – Recovery Process Flow Chart



Appendix N: Job Aid 6 – Entry to Care Monitoring and Evaluation

ENTRY TO CARE PROTOCOL

Monitoring & Evaluation

Indicator	Numerator	Denominator
Percentage of persons diagnosed with HIV not linked to treatment and care services.	The number of people who are not linked to any treatment and care services. *Data Source: Laboratory Data Collection Tool cross checked with TSIS*	The number of persons diagnosed with HIV *Data Source: Laboratory Logbooks*
Percentage of PLHIV unlinked who are locatable	The number of PLHIV unlinked with sufficient locatable information. *Data Source: Line Listing of PLHIV not linked to care cross checked with CI Field notes/record*	The number of people who are not linked to any treatment and care services *Data Source: Laboratory Data Collection Tool cross checked with TSIS*
Percentage of PLHIV located	The number of unlinked PLHIV located. *Data Source: Line Listing of PLHIV not linked to care*	The number of PLHIV unlinked with sufficient locatable information. *Data Source: Line Listing of PLHIV not linked to care cross checked with CI Field notes/record*
Percentage of PLHIV linked of those who were not linked to treatment and care services	The number of PLHIV successfully linked to care. *Data Source: Line Listing of PLHIV not linked to care*	The number of PLHIV unlinked who were located and alive. *Data Source: Line Listing of PLHIV not linked to care*
Number of unlinked PLHIV with completed investigations	The number of cases that received a final disposition. *Data Source: Line Listing of PLHIV not linked to care*	N/A
Number of unlinked PLHIV with unresolved cases	The number of cases without a final disposition *Data Source: Line Listing of PLHIV not linked to care*	N/A

Appendix O: Job Aid 7 – Linkage and Retention Monitoring and Evaluation

Retention and Recovery PROTOCOL

Monitoring & Evaluation

Indicator	Numerator	Denominator
Percentage of PLHIV with missed appointments	The number of people who did not visit the clinic on their recorded next appointment date *Data Source: TSIS, Patient Contacting Tracking Tool*	The number of persons with a next appointment date in the reporting month *Data Source: TSIS*
Percentage of PLHIV with missed appointments rescheduled	The number of missed appointment cases that were reached and have a rescheduled appointment date *Data Source: Patient Contacting Tracking Tool*	The number of people who did not visit the clinic on their recorded next appointment date *Data Source: TSIS, Patient Contacting Tracking Tool*
Percentage of PLHIV who attended rescheduled appointment date	The number of PLHIV who attended their rescheduled appointment date after missing their appointment in a given month. *Data Source: TSIS, Patient Contacting Tracking Tool*	The number of missed appointment cases that were reached and have a rescheduled appointment date. *Data Source: Patient Contacting Tracking Tool*
Percentage of PLHIV retained on ART	The number of PLHIV with an active prescription OR a prescription end date no more than 28 days past. *Data Source: TSIS*	The number of PLHIV registered in the health facility *Data Source: TSIS*